CHAPTER THREE

The Chimera of the Curative Asylum

> Madmen appear to have been employed to torment other madmen, in most of the places intended for their relief.
>
> Samuel Tuke, *Description of the Retreat*

> Tenderness is better than torture, kindness more effectual than constraint. . . . Nothing has a more favourable and controlling influence over one who is disposed to or actually affected with melancholy or mania, than an exhibition of friendship or philanthropy.
>
> John Reid, *Essays on Hypochondriasis and Other Nervous Affections*

> I want to stir up an intelligent and active sympathy, in behalf of the most wretched, the most oppressed, the only helpless of mankind, by proving with how much needless tyranny they are treated – and this in mockery – by men who pretend indeed their cure, but who are, in reality, their tormentors and destroyers.
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> John Perceval, *A Narrative of the Treatment Experienced by a Gentleman, During a State of Mental Derangement*

1. The 1815–16 Parliamentary Inquiry

The new Committee included several of those who had been members of the 1807 investigation,[1] but the inquiry which ensued was quite unlike its predecessor. Lasting almost two years and encompassing a whole series of detailed reports and minutes of evidence, the inquiry surveyed treatment in charity hospitals (Bethlem, York Asylum, St. Luke's); in the new county asylums (Nottingham, one of only three open at this time); in private madhouses (a number, ranging from Warburton's houses at Bethnal Green and Spencer's house at Fonthill Gifford at one extreme, to Brislington House and Laverstock at the other); and in workhouses. Forty-one witnesses testified before it, including fifteen members of the medical profession (ten physicians, four surgeons, and an apothecary); twelve madhouse keepers; and an assortment of magistrates, members of

1. These included the chairman of the Committee, George Rose (a close personal friend of George III before the latter's descent into madness and dementia); and Charles Williams-Wynn, Shaw Lefevre, and Samuel Whitbread.

the Committee itself, and various 'Gentlemen, who, from motives of benevolence, had given particular attention to inquiries connected to this subject'.[2] In addition, some of those interested in influencing the course of future legislation submitted written evidence, subsequently published in pamphlet form.[3]

The reformers were now sure of what they wanted to achieve. The focus throughout their inquiry was on the conditions endured by those insane confined in institutions, with no effort being made to gather comparable data on lunatics in the community. Yet although abuses and maltreatment of patients were found to be prevalent in virtually every type of institution they examined, both the Committee itself and those who disseminated its findings to a wider public[4] interpreted these revelations as proof of the need for more institutions (albeit under direct public control), as well as justifying (indeed making imperative) an improved system of inspection and supervision of all receptacles within which lunatics were confined. And there can be no question but that, through the impact of its portrait of the plight of the insane in existing institutions, this inquiry helped to create a climate of opinion favouring reform.

The early meetings of the Committee were largely taken up by an investigation of the recent events at the York Asylum and at Bethlem. The reformers realized that this material provided some of their most powerful ammunition against the existing system. Accordingly, they were careful to avoid all appearances of bias, offering those criticized the opportunity to rebut the testimony given against them, while making sure that the abuses and depredations earlier investigations had uncovered received a thorough airing. The reformers' case was overwhelming. At York, as we have seen, they had already succeeded in forcing the resignation of the asylum physician, Dr Best, had dismissed the staff, and had undertaken a total reorganization of the entire institution. Higgins, who had played such a prominent role in these affairs, was the Committee's first witness. He placed on record an extensive narrative of events prior to reform, including the frantic efforts of the asylum staff to conceal their wrong-doing by the destruction of records and, in all probability, by deliberately burning down a large part of the asylum building itself – a fire in which at least four patients were acknowledged to have died. His old adversary, Dr Best, was, therefore, already discredited by the time he appeared before

2. [W.H. Fitton], 'Lunatic Asylums', *Edinburgh Review* 28, 1817, p. 451.
3. For example, Thomas Bakewell, *A Letter Addressed to the Chairman of the Select Committee . . . Appointed to Enquire into the State of Madhouses*, Stafford: for the author, 1815; William Charles Ellis, *A Letter to Thomas Thompson, Esq., M.P., . . . on the Necessity of Proper Places Being Provided by the Legislature for the Reception of All Insane Persons*, Hull: Topping and Dawson, 1815; William Nisbet, *Two Letters to the Right Honourable George Rose, M.P., . . . on the State of the Madhouses*, London: Cox, 1815.
4. See esp. [W.H. Fitton], 'Lunatic Asylums', pp. 431–71; 'Insanity and Madhouses', *Quarterly Review* 15, 1816, pp. 387–417.

the Committee, and the mixture of bluster and outright denial he exhibited on the witness stand failed to rescue his reputation.[5]

The situation at Bethlem was rather different. In the aftermath of Wakefield's first visit, and his return with a party of MPs, the governors had felt compelled to institute their own inquiry into the conduct of their officers and the condition of the asylum. Incredibly, from the reformers' perspective, they had convinced themselves that the charges were without foundation, that the institution was as good as, if not better than, any in England, and that the conduct of the physician and apothecary deserved praise rather than opprobrium. Indicative of their attitude was their verdict on the case of Norris: his confinement, they solemnly averred, had been 'upon the whole, rather a merciful and a humane than a rigorous and severe imposition'.[6]

Consequently, the reformers were here concerned not merely with placing on the public record an account of past mismanagement, but also with provoking a more suitable response from the governors. Wakefield's evidence itself provided a damning indictment of the asylum's administration; but, in addition, several of the MPs had themselves witnessed the abuses he complained of on their own tour of Bethlem. When the apothecary and the physician, Haslam and Monro, appeared before the Committee, their position, following their recent vindication by their employers, was ostensibly secure. The hostile cross-examination which they now faced, however, forced both of them, first into evasion and denial, and then into self-contradiction and damaging admissions about their own conduct in office. Finally, in desperation, each man resorted to blaming the other, or the surgeon, Bryan Crowther, who had conveniently died earlier that year. In the words of a contemporary, the impression they left was of 'a physician who walked the hospital only once a month – an apothecary, who abounded in theoretical views, but was above attending to anything else – a steward, a matron, and a porter, all too important in their own eyes to attend to the wants and necessities of the patients – and a surgeon often mad himself, and almost continually drunk'.[7]

As soon as their first report was printed, the Committee ordered copies to be sent to every governor of Bethlem, a scarcely veiled suggestion that

5. For Higgins' evidence, see Select Committee Report, 1815, pp. 1–9; his evidence was corroborated by Bryan Cooke, a fellow magistrate and governor of the York Asylum, *ibid.*, p. 9. For Best's reply, see *ibid.*, pp. 144–7. Higgins subsequently published an annotated version of the testimony before the Committee on the events at the York Asylum, sarcastically refuting Best's defence point by point. See Godfrey Higgins, *The Evidence Taken Before a Committee of the Houses of Commons Respecting the Asylum at York, With Observations and Notes*, Doncaster: Sheardown, 1816.
6. *Report from the Committee of Governors of Bethlem Hospital [on] the Case of James Norris*, reprinted as an appendix to Select Committee, *Report*, 1815. Their bluster was rather undermined by their simultaneous order that he be released, and by Norris's own inconvenient decision to die a few weeks thereafter.
7. *Hansard's Parliamentary Debates*, Vol. 34, first series, 1816, col. 426.

the latter should reconsider their earlier action. In the face of this pressure, the governors convened a special meeting and called the physician and the apothecary before them for a second time. Each arrived with an air of injured innocence and presented the governors with a written defence of his conduct, pointing out that the charges against them were the very same ones the governors themselves had concluded were baseless only a few months previously.[8] The governors proved no less adept than their employees at contradicting themselves. Haslam was dismissed, and within six weeks, they accepted Monro's resignation.[9]

In the meantime, the Committee's inquiries had moved from a concern with conditions in the principal charity hospitals of the period[10] to a consideration of the treatment meted out to those confined in workhouses and in a number of private institutions avowedly operated for profit. Much of the testimony on these places came from a handful of reformers who appear to have made almost a full-time vocation of inspecting the conditions under which lunatics were kept. Wakefield, for example, in his travels as a land-agent, had inspected a number of provincial madhouses. A few, such as Brislington House and Laverstock House, were exceptions to the general trend, and failed to add to the catalogue of abuses the Committee was now uncovering. At Langworthy's House at Box in

8. John Haslam, *Observations of the Apothecary of Bethlem Hospital Upon the Evidence Taken Before the Committee of the Honourable House of Commons for Regulating Madhouses*, London: Bryer, 1816; Thomas Monro, *Observations of Dr Monro Upon the Evidence Taken Before the Committee of the Honourable House of Commons for Regulating Madhouses*, London: Bryer, 1816.
9. That this sequence of events was little more than a tactical concession is suggested by the governors' choice of a physician to succeed Thomas Monro, his son, Edward Thomas Monro.
10. The Committee secured some evidence on the lunatic ward at Guy's Hospital, and on St. Luke's. Dunston, the keeper at the latter institution, was allowed to offer uncontroverted evidence that each patient in his establishment was well fed and given a proper wooden bed, and that restraint was kept to a minimum – he claimed only five of three hundred patients were under restraint at the time of his testimony. The Committee's unusual passivity may have reflected the presence of two Governors of St. Luke's (Shaw Lefevre and Whitbread) among its own membership. Kathleen Jones (*Lunacy, Law, and Conscience, 1744–1845*, London: Routledge and Kegan Paul, 1955, pp. 96–8) has echoed the claim that St. Luke's was better than Bethlem or the York Asylum, but contemporary evidence suggests otherwise. Wakefield testified before the Committee itself that he had found the place overcrowded and cheerless, with unglazed cells, patients chained to their beds, nearly naked and clad only in rags (Select Committee, *Report*, 1815, pp. 16–7). William Tuke's notes of a visit in 1812 (reprinted in D.H. Tuke, *Chapters in the History of the Insane in the British Isles*, London: Kegan Paul and Trench, 1882, pp. 89–90) paint a similar picture: 'There are about three hundred patients, sexes about equal. . . . The superintendent has never seen much advantage in medicine and relies chiefly on management. Thinks chains a preferable mode of restraint to straps or the waistcoat in some violent cases. Thinks confinement or restraint may be imposed as a punishment with some advantage, and, on the whole, thinks fear the most effectual principle by which to reduce the insane to orderly conduct. The building has entirely the appearance of a place of confinement . . . and there are strong iron grates to the windows.' It seems clear that under other circumstances, St. Luke's would have been cited as another example of the need for reform.

Wiltshire, however, the pattern was more typical: he was refused permission to see male patients on the grounds that this was one of the days they were not allowed up. Among the women, he found two lying naked on straw pallets, and four others left entirely naked in total darkness: 'in the course of my visiting these places, I never recollect to have seen four living persons in so wretched a place.'[11] Back in London, he attempted to view Sir John Miles's madhouse in Hoxton, the largest of the metropolitan establishments. Once again, Wakefield was refused admission, the keeper conceding that to allow 'an inspection of that house would be signing its death warrant'.[12]

The most extensive evidence on the treatment of the insane confined in workhouses was provided by Henry Alexander, a banker who had developed an interest in lunacy reform. Like Wakefield, he had taken advantage of the travelling he had to do for business purposes to undertake a series of unofficial inquiries into the conditions of the insane. Some he had found in rooms 'no better than dungeons . . .'; elsewhere, the cells were 'like pigstyes'. His description of the conditions at the Tavistock Workhouse was particularly graphic. He was refused permission to see the lunatics themselves, and when he persisted, the master reluctantly showed him the quarters they were usually housed in, first warning him that he would find them unfit to enter, even though they had been washed out earlier that morning:

> I have never smelt such a stench in my life, and it was so bad, that a friend who went with me [into the first cell] said he could enter the other. After having entered one, I said I would go into the other; that if they could survive the night, I could at least inspect them. . . . The stench was so great I almost suffocated; and for hours after, if I ate anything, I still retained the same smell; I could not get rid of it; and it should be remembered that these cells had been washed out that morning, and had been opened some hours previous.[13]

11. House of Commons, *Report of the Select Committee on Madhouses*, 1815, p. 21.
12. *Ibid.*, p. 19.
13. *Ibid.*, p. 3. To some extent, of course, this and similar references to filth and stench reflect (and contribute to) what Corbin has termed an 'olfactory revolution', a mounting intolerance among the better sort for the dirt and foul odours which had been so much a part of organized social life in previous centuries. See Alain Corbin, *The Foul and the Fragrant: Odor and the French Social Imagination*, New York: Berg, 1986, esp., pp. 57–85 and 89–110. (Originally published as *Le miasme et la jonquille*, Paris: Aubier Montaigne, 1982.) Jonathan Andrews has similarly emphasized that 'Calls for hygiene grew more insistent, not merely out of developing theories concerning the generation of putrid distempers, but alongside a growing sensibility and insistence upon outward decorum, which began to challenge former assumptions that the squalid conditions of the poor, sick and insane were unavoidable, or even appropriate, and sought to impose cleanliness as (next to godliness) one means of subjugating the signs and symptoms that had marked these groups out so visibly as disordered.' 'Bedlam Revisited: A History of Bethlem Hospital *c.* 1634–*c.* 1770,' unpublished Ph.D. dissertation, London University, 1991, pp. 164–5.

The Committee next sought evidence on the operations of the system for inspecting metropolitan madhouses under the 1774 Act, a task which had been delegated to members of the Royal College of Physicians. Dr Richard Powell, who had acted as the secretary of the Commissioners since 1808, was their principal witness. There were thirty-four madhouses under the Commissioners' direct jurisdiction, and Powell testified that on their tours of inspection they visited 'some days, perhaps two; other days six or eight' houses within the space of a few hours. Though they suspected some houses of concealing the actual number of patients crammed into their overcrowded rooms, they made no effort to see that the legal requirement that all inmates be reported to them was observed, or to inquire into the disposition of individual cases. Given the brief duration of the average inspection, such procedures were totally impracticable. At the most recent visit to Miles's establishment at Hoxton, for example, the inspection of the whole structure and the condition of its 486 inmates had been completed within two and a half hours.[14] The lunacy reformers on the Committee were naturally aware that even had the Commissioners bothered to uncover evidence of systematic brutality and neglect, the latter lacked any legal power to intervene. But, of course, this only strengthened their resolve to replace the existing system altogether.

At the close of the inquiries in 1815, therefore, the Committee had uncovered evidence that neglect and maltreatment of lunatics was endemic in all the various types of institutions in which they were incarcerated, and its members were convinced that legislative intervention was urgent. Fearful perhaps that others might not be so readily convinced, they continued their investigations into the following session, uncovering evidence which suggested massive irregularities in two of the largest and best-known private madhouses in London, Warburton's Red and White Houses at Bethnal Green. John Rogers, the apothecary at the White House, gave evidence which suggested a state of affairs at least as sordid as that revealed at Bethlem and at the York Asylum. The houses themselves, he alleged, were infested with rats and fleas, and were so cold and damp that many patients suffered from gangrene and tuberculosis. At least one patient had had to have both feet amputated when mortification set in, and as many as a hundred patients had died in the winter of 1810–11 from typhus. Instances of gross brutality inflicted on patients by the keepers went unpunished even where they caused a patient's death. Female patients were liable to be raped. Beating and whipping were common punishments and prolonged restraint was employed as a routine means of managing patients. Incontinent patients were 'mopped down' in a none-too-gentle

14. House of Commons, 1815, p. 76. In view of Wakefield's earlier testimony, the Committee can scarcely have been reassured to learn that Powell considered Miles's house to be kept 'in very excellent order'.

fashion under a stream of cold water from a pump in the yard. Refusal to eat or to take prescribed medications was met by force: 'I have known sundry instances where the mouth had been lacerated and the teeth have been forced out. I recollect Mrs Hodges, wife of the vestry clerk at St. Andrew's Holborn, dying in this way. I do not suppose there is a keeper who has been in these houses four or five years who has not had patients die under their hand in the act of forcing.'[15]

Rogers reiterated these and other allegations in a pamphlet he had published at his own expense.[16] The detailed nature of the evidence he gave and his willingness to cite the names and dates of actual cases made him an impressive witness, and his sister, Mary Humieres (who had previously served as a housekeeper at the White House), returned from Switzerland to corroborate his accusations. Their testimony was, quite naturally, vigorously contested by Warburton and his minions. Matthew Talbot, superintendent of the White House, accused Rogers of deliberate falsification, and Warburton himself appeared before the Committee to issue a blanket denial of all the charges: 'I never knew of an instance of an injury so much as any boy has received at school from a chilblain, or not more.'[17] Unfortunately, the Committee's failure to investigate conditions at first hand left the issue of who was telling the truth unresolved, though the mere suspicion that such events might have occurred was enough for many of the reformers.[18]

By the time the 1815–16 Committee submitted its final report, there was a wealth of documentation to support the reformers' contention that what they perceived as appalling degradation and inhuman treatment were the lot of madmen in every sort of institution in which they were confined. Future investigations might (and did) add further examples of these selfsame abuses inflicted on other madmen in other asylums; but the fundamental picture was already clear, and the evidence sufficiently abundant to convince most observers of the justice of the reformers' claims about conditions in existing asylums, if not the correctness of their prescription for reform. Likewise, the reformers were already convinced that they knew what had to be done to cure the 'intolerable' evils they had

15. House of Commons, *First Report of the Select Committee on Madhouses*, 1816, pp. 7ff.
16. J.W. Rogers, *A Statement of the Cruelties, Abuses, and Frauds, Which Are Practised in Mad-Houses*, London: for the author, 2nd edn, 1816.
17. Select Committee, *First Report*, 1816, pp. 23, 38.
18. Subsequent events tended to confirm Rogers's veracity. In 1825, John Mitford published a two-part exposé of *The Crimes and Horrors of Warburton's Private Madhouses* (London: Benbow, 1825), in which he claimed that the keepers' motto was 'If a man comes in here mad we'll keep him so; if he is in his senses, we'll soon drive him out of them.' Less polemically, the proceedings of a subsequent Parliamentary inquiry in 1827 focused directly on Warburton's establishments, and provided detailed evidence in support of Rogers's allegations, some eleven years after they were first made. Rogers himself reappeared as a witness in these hearings, contending that he had been ruined by the machinations of private madhouse keepers after his previous testimony. See House of Commons, *Report of the Select Committee on Pauper Lunatics*, 1827, pp. 144–6.

exposed. There were two central elements in their plan: the provision of a system of asylums at public expense to take at least all of the pauper lunatics ought to be made compulsory, in order to eliminate the inducements to maltreatment inherent in keeping lunatics for profit;[19] and a vigorous system of inspection by outsiders with no ties to the asylum administration should be implemented, to provide a check against the tendency of all institutions to fall away from their initial ideals and against the temptations for the keepers to neglect and maltreat their helpless 'clients', the mad.[20] These two proposals remained the fundamental features of the reform plan until its eventual implementation. Yet despite the obvious propaganda value of the Committee's revelations in producing converts to the reformers' cause, it took the latter thirty years to attain their objectives.

2. *The Fate of the First Reform Bills*

In the aftermath of the Committee's findings, the reformers sought, in the first instance, to replace the existing moribund provisions for the inspection of madhouses with the kind of efficient national inspectorate which they considered an essential prerequisite for reform. Three times between 1816 and 1819, members of the Select Committee presented bills to introduce such a provision. Each bill proposed a permanent commission of eight, to be appointed by the Home Office, with power not merely to inspect private madhouses, but also to lay down and enforce standards for the management of these institutions and the treatment of patients. On each occasion, the Parliamentary reformers were able to make use of the massive evidence of abuse which they had uncovered, and the manifest inadequacies of the existing system of inspection, to secure majorities in the House of Commons for the bill's passage; on each occasion the measure was rejected in the Lords.

What accounts for these failures? In the first place, it is clear that the reformers made some tactical errors which hurt their cause. For example, in 1816, when the revelations of their Committee were still fresh in the public mind, the reformers made the error of including in their bill a provision for the stringent inspection of 'single lunatics'. This was a measure known to offend the aristocratic sensibilities of their Lordships, who were concerned to protect the privacy of upper class families with a

19. Samuel Tuke had suggested this measure in an article in *The Philanthropist* ('Essay on the State of the Insane Poor', 1, 1811, pp. 357–60), and it was vigorously advocated by W.C. Ellis in his written submission to the Committee. Its necessity was impressed on a wider public by the *Quarterly Review* (15, 1816, p. 416).

20. Again, this was a central theme in Samuel Tuke's writings (see both the *Description of the Retreat: An Institution Near York for Insane Persons of the Society of Friends*, York: Alexander, 1813, and his pamphlet, *Practical Hints on the Construction and Economy of Pauper Lunatic Asylums*, York: Alexander, 1815, reemphasized by the *Quarterly Review* 15, 1816, p. 415).

lunatic in the closet, and it undoubtedly contributed to the bill's defeat. Subsequently, this provision was so watered down in the 1817 bill as to be almost meaningless, but by then the reform movement was losing its momentum. The *Edinburgh Review* sought to rally support for the measure, urging that while 'many of the evils have already ceased to exist in some of the principal institutions for the insane . . . they were the offspring of circumstances that, without a radical change of system in the control of madhouses, cannot fail to produce again the same deplorable effects'.[21] By now, however, opponents of the bill were exerting pressure in the opposite direction.

Such opposition had a number of sources. Most obviously, it came from those whose interests in the mad-business were directly threatened by the new schemes. Madhouse proprietors complained that though the Committee's findings had 'excited a very high degree of public feeling', the testimony it had heard was not given on oath,[22] 'and, whilst it exaggerates praise in a few instances, bestows much undeserved censure in others'. The publication of such evidence was 'likely to be productive of positive injury in various ways, both to society and to individuals. Society at large must suffer if insane persons are allowed to range uncontrolled, whilst they are not regulated by reason in their conduct, and are morally irresponsible as they should therefore be, for their actions'. In consequence of the prejudice against madhouses the reformers had stirred up, 'it does indeed so suffer at present, and crimes which are referable to insanity are largely increased'.[23] In a more subtle kind of special pleading, which we will discuss at greater length in Chapter 4, the reformers' proposals were pictured as an assault on medicine's professional prerogatives, as encouraging lay interference in technical decisions about the proper treatment of the insane, and hence as productive of serious damage to the patients' best interests.[24]

One doubts, though, whether such self-interested sophistry would have carried the day but for the existence of a much more broadly based and politically powerful source of opposition to the reformers' plans. The critical strength of the resistance movement came from its links with the localist bias characteristic of English society until well into the nineteenth century. This aversion to the concentration of power at the national level was extraordinarily widespread and well-entrenched on both the structural and ideological levels. Undoubtedly, its origins lay in the gentry's successful resistance to Stuart attempts to impose an indigenous English form of absolutism (though such convictions concerning the threat which

21. [W.H. Fitton], 'Lunatic Asylums', p. 434.
22. George Man Burrows, *Cursory Remarks on a Bill now in the House of Peers for Regulating of Madhouses*, London: Harding, 1817, p. 1.
23. Anon., *Insanity*, London: Underwood, 1817, p. 5.
24. An earlier effort to strengthen the system for inspecting madhouses had also been stifled, in part at least, by medical opposition. See *Medical and Surgical Journal*, 1814, pp. 1–2.

centralized state power offered to the liberties and freedom of a propertied class had received more recent and dramatic confirmation in the French Revolution and its aftermath). The English upper class had secured its power by ensuring that administration remained locally based, and by convincing the common people (as well as themselves) that an increase in the authority of the state threatened to produce an engine of despotism, 'a system of tyranny . . . , the destruction of all public liberty, and the disturbance of all private happiness'.[25] The hegemony of these local landed elites, astutely legitimized through the legal system and finding practical expression in the powers of the magistrates and the innumerable autonomous local authorities, did not simply melt away at the first demands for a more rational system.[26] On the contrary, in the first decades of the nineteenth century these administrative units jealously defended their independence against the Benthamites and the rising class of professional administrators.[27]

The effort to rewrite the legal rules concerning insanity and the treatment of the insane was necessarily productive of opposition on this score, since it involved interference with the 15,000 or so local Poor Law administrations which had previously possessed untrammeled authority to dispose of the insane (along with the rest of the indigent) with virtually total freedom from central supervision and control. The reformers' proposals for a system of inspection by paid officials dependent upon the central government – officials who would have had the power to enforce centrally determined directives and standards – threatened a transformation in political relationships whose importance extended far beyond the narrow sphere of lunacy reform. In securing acceptance of this type of administrative apparatus, the central government would have obtained far greater power than it had hitherto possessed to ensure compliance with its wishes – with a corresponding setback to the power of the landowning class.

The political leverage of the rural aristocracy – locked in a losing battle with the rising manufacturing and commercial bourgeoisie – was by now clearly on the wane. As economic life – indeed, life in general – became

25. J.P. Smith (1812), quoted in E.P. Thompson, *The Making of the English Working Class*, New York: Vintage, 1963, p. 82.
26. The impact of these structural features of the English political system is vividly demonstrated in the long-running conflict over the rationalization of the English criminal law. See the discussions in Douglas Hay, Peter Linebaugh, and E.P. Thompson (eds) *Albion's Fatal Tree: Crime and Society in Eighteenth Century England*, New York: Pantheon, 1975; and in Steven Spitzer and Andrew Scull, 'Social Control in Historical Perspective: From Private to Public Responses to Crime', pp. 281–302 in D.F. Greenberg (ed.) *Corrections and Punishment*, Beverly Hills, California: Sage, 1977. Opposition to the further concentration of power at the national level was reflected in such actions as the rejection of Bentham's plans for a 'Ministry of Police' by an 1818 Parliamentary committee, and perhaps reached its apogee in the resistance to the passage and implementation of the Poor Law Amendment Act of 1834.
27. See generally, William C. Lubenow, *The Politics of Government Growth: Early Victorian Attitudes Toward State Intervention 1833–1848*, Newton Abbot: David and Charles, 1971.

increasingly oriented towards and dictated by conditions in national and international markets, so the significance of the purely local and personal dependencies to which the authority of the traditional elites was bound steadily declined in significance. Ever more aware of the advantages of impersonal forms of coercion – most especially the invisible hand of the marketplace – the bourgeoisie were in consequence ever less disposed to rely on the visible and personal authority of an earlier era. And yet, while capitalism dissolved traditional social restraints, capitalists as a class sought a previously unknown degree of stability and predictability in social relationships, for the market was extraordinarily sensitive to disorder and uncertainty. In the long run, only a greatly strengthened state apparatus could provide such guarantees, particularly on a national basis.

In the short run, however, the position of the gentry remained sufficiently entrenched to ensure that the old habits of thought and existing systems of political organization would be defended with much of their old vigour. Two additional factors stiffened resistance to the lunacy reformers' proposals. The plan to make construction of county asylums compulsory threatened local authorities with heavy capital expenditures which they were reluctant to incur, particularly since they faced simultaneous pressures to provide new jails and workhouses, and to develop a communications network adequate for the emerging industrial society. Then, too, the uneven spread of the new outlook on the insane left many sceptical of the need for change. Local magistrates continued to accept the traditional paradigm of insanity, along with its emphasis on the demonological, almost bestial character of madness. Consequently, they were frequently unable to comprehend why the reformers saw the treatment of lunatics within their jurisdictions as brutal and inhumane; why conditions they saw as unexceptionable produced shock and outrage in others. Not surprisingly, then, it was only after the manufacturing and commercial bourgeoisie had secured a substantial increase in their political power and influence through the 1832 reform of Parliament that the obstacles to central administration and direction were confronted and dealt a decisive defeat; and the issue was fought out, not over the marginal issue of lunacy reform, but over the crucially important matter of Poor Law policy – the Old Poor Law being the single most important remaining limitation on the free market in labour.

3. *Renewed Parliamentary Investigation*

The defeat of the 1819 bill led to a brief hiatus in the effort to secure legislative implementation of the lunacy reformers' plans. Several leading parliamentary supporters had died, while others had retired.[28] In 1827,

28. Whitbread had committed suicide in 1815, and Romilly in 1818. George Rose died of old age, also in 1818, and shortly thereafter the aging Lord Robert Seymour had given up his Parliamentary seat.

however, yet another parliamentary inquiry re-awakened interest in the issue. Between that date and 1845, when the major elements of the reform program were enacted into law, the reformers' efforts to overcome their opposition proceeded on two fronts. The first and most visible of these was the political: they made use of astute parliamentary manoeuvering and a further series of official inquiries to keep the issue before the public and eventually to wear down their opponents. As we shall see, the success of this first tactic was intimately tied to activity of a rather different sort – the development over the second quarter of the nineteenth century of an increasingly elaborate pro-institutional ideology designed to rally public support for the lunacy reformers' plans.

The revival of parliamentary interest in lunacy reform came about through the persistence of Lord Robert Seymour, a member of the 1815–16 Committee who had subsequently given up his seat in Parliament. In his remaining public capacity, as a magistrate for the county of Middlesex, he had sought for years to persuade his reluctant fellow magistrates of the need for a county asylum. Finally, in 1827, they were prevailed upon to set up a committee of magistrates to inquire into conditions in the private madhouses in the county. Armed with the evidence produced by a visit of the overseers of the poor for St. George's parish to one of Warburton's houses, Seymour also approached those interested in lunacy reform in the Commons, and pressed for a renewed inquiry into conditions in the metropolitan madhouses. Here again he was successful: on 13 June 1827, Robert Gordon, a fellow magistrate and MP for Dorset, secured the establishment of a new Select Committee charged with this task.

For the most part, the Committee confined its attention to a detailed inquiry into conditions at Warburton's White House in Bethnal Green, taking extensive testimony from magistrates and Poor Law officers who had visited it to inspect the conditions of the pauper patients, from members of the Royal College of Physicians who had inspected it under the provisions of the 1774 Madhouse Act, and also from a number of ex-patients. Almost without exception, their evidence revealed a pattern of systematic abuse, maltreatment, and neglect of the inmates. On the first day of the hearings, John Hall, a guardian and director of the poor for the parish of Marylebone, gave evidence about a visit he had made in August of the previous year. Informed that one of the lunatics from his parish was ill and was therefore being cared for in the infirmary, he had brushed aside the objections raised by the keeper, and had insisted on examining him:

> we found a considerable number of very disgusting objects – a description of pauper lunatics I should conceive chiefly idiots, in a very small room, and several of them were chained to the wall. The air of the room was highly oppressive and offensive, insomuch so that I could not draw my breath. I was obliged to hold my breath while I staid to take a very short survey of the room. . . . It contained the description of

> patients called the wet patients; they were chiefly in petticoats . . . they appeared to be of the worst description of decided idiots; and the room was exceedingly oppressive from the excrement and smell which existed there.[29]

The evidence of other witnesses made it clear that those who had the good fortune to avoid the special care provided for the physically sick were scarcely better off. A large number of the 'patients' were kept confined in 'crib rooms', places where

> there are nothing but wooden cribs or bedsteads; cases, in fact, filled with straw and covered with a blanket, in which these unfortunate beings are placed at night; and they sleep most of them naked on the straw, covered with a blanket . . . they are mere boxes of the depth of about eighteen inches, where the person lies in; they are all fastened; some all fours, some one, some two, and some three [limbs].[30]

Fifteen of these cribs were crammed into a single room, twenty-nine feet by fifteen. Each day patients were locked into their cribs 'about three o'clock in the afternoon, and . . . did not get up until nine in the morning'.[31] To allow the keepers a day off one day a week

> on Saturday evenings, they were locked down in the same state, and kept till Monday morning, without being unchained or allowed to get up to relieve themselves in any way whatever. . . . On Monday morning, like the other mornings, when they got up, they were many of them in a very filthy state, and I have seen them, when the snow has been upon the ground, put into a tub of cold water and washed down with a mop; there was a man from Northamptonshire, who was treated in that way; I have seen that man brought up from the door of the room, and from the heat of the faeces that were lying on him, his back has been completely bare for many inches up, and he was treated in the same way, by being washed in the way I have stated.[32]

The cribs themselves were generally in a filthy state: 'I turned the straw out of some of the cribs and there were maggots at the bottom of them where sick men had laid.'[33]

No effort was made to separate patients according to the degree of their disorder, cases of all descriptions congregating in the single exercise yard

29. House of Commons, *Select Committee on Pauper Lunatics in Middlesex*, p. 15.
30. *Ibid.*, pp. 30–1, evidence of Richard Roberts.
31. *Ibid.*, pp. 37, 98, evidence of John Nettle, and of Thomas Jennings, Warburton's head keeper.
32. *Ibid.*, p. 32, evidence of William Soloman. This account was corroborated by several other witnesses. See *ibid.*, pp. 37, 38, 40, 128.
33. *Ibid.*, p. 37, evidence of John Nettle.

allotted inmates of each sex. The visitor would find 'some of them chained to seats, and some of them handcuffed; and there are some exceedingly noisy, and others melancholy and dull, all in the same apartments, and in a yard too small for such a number, where they have not room for exercise or for employment. . .'.[34] Patients necessarily performed most of the work in the asylum – on the men's side, for instance, there were only two paid attendants for 164 male pauper lunatics.[35] Warburton himself put in an appearance twice a week, as did Thomas Jennings, the man nominally in charge of the day-to-day administration of the establishment.[36]

As the Committee took pains to document,[37] such conditions were scarcely atypical of those metropolitan madhouses which concentrated on the pauper lunatic trade, and the reformers now made use of this ostensibly local investigation to reopen the issue of legislation to give effect to at least some of the changes they considered necessary. This time the bills were framed to avoid offending the local gentry and magistrates, and to reduce the most obvious manifestations of central control to a minimum, so as to eliminate the most important source of opposition to the earlier bills. The bill which eventually became the County Asylums Act (9 George IV *c.* 40) on the surface added very little to previous acts regulating these establishments. It did, however, create a precedent for some central government interest in the internal affairs of country asylums through the apparently trivial provisions that the magistrates were to send annual returns of admissions, discharges and deaths to the Home Office, and that the Secretary of State could, if he so chose, send a visitor to any county asylum – though such a visitor had no legal power to intervene in asylum administration. A second bill, which passed as the Madhouse Act (9 George IV *c.* 41), marked another partial success for the reformers. The 1827 inquiry had again shown that inspection on a part-time basis by the College of Physicians under the 1774 Madhouse Act was little more than a cosmetic measure with no practical force,[38] and with this new act they

34. *Ibid.*, p. 24, evidence of Garrett Dillon, a surgeon.
35. *Ibid.*, p. 34, evidence of William Soloman.
36. *Ibid.*, p. 34, evidence of William Soloman.
37. See the evidence of Garrett Dillon, Dr John Bright, Peregrine Fernandez, and Dr Alexander Franston, and the detailed evidence on the state of Casey's, Burrows's, and Holt's madhouses, House of Commons, *Report on Pauper Lunatics in the County of Middlesex*, pp. 153–8. At the latter, for instance, 'In an outhouse at the bottom of the yard, ventilated only by cracks in the wall, were enclosed three females, the door was padlocked; upon an open rail-bottomed crib herein, without straw, was chained a female by the wrists, arms, and legs, and fixed also by chains to the crib; her wrists were blistered by the handcuffs; she was covered only by a rug; the only attendant upon all the lunatics appeared to be one female servant, who stated she was helped by the patients.'
38. For discussion of the role of the College of Physicians in inspecting asylums before 1828, see Nicholas Hervey, 'A Slavish Bowing Down: the Lunacy Commission and the Psychiatric Profession 1845–60', in W. Bynum, R. Porter, and M. Shepherd (eds), *The Anatomy of Madness*, Vol. 2, London: Tavistock, 1985, pp. 99–101. Hervey documents the close personal and professional links between the physicians inspecting the madhouses and the madhouse keepers themselves, and their total failure to establish any corpus of administrative expertise.

finally secured its abolition and replacement. To achieve this, however, they were forced to engage in a temporary tactical retreat from the proposals for a full-time national 'Board of Inspection' which had been so prominent a feature of the 1816, 1817, and 1819 bills, and which remained the solution they ultimately wanted to implement.[39]

The 1828 Act did provide for the establishment of a new Commission to license and supervise lunatic asylums. Its activities were, however, to be confined to the metropolitan area. Moreover, the Commissioners were to be appointed on a purely part-time basis, with the five physician Commissioners receiving token payment, and the rest giving their services gratuitously. In this respect it represented a reversion to the amateur system of the past, an apparent abandonment of the earlier insistence on the Benthamite ideal of full-time, expert administrators; and in filling the medical positions on the Commission, the Home Secretary, Peel, deferred to Sir Henry Halford, President of the Royal College.[40] Meanwhile, in the provinces, in what was basically but a minor departure from the provisions of the 1774 Act, the duties of visitation and licensing remained in the hands of the magistrates – three justices and a medical attendant were to be appointed at the General Quarter Sessions and charged with visiting all houses licensed to receive lunatics at least three times a year. Finally, the Act's provisions for inspection applied only to private madhouses, not to county asylums, or to the charity or 'subscription' asylums.

In all these respects, the bill to regulate madhouses presented in 1828 seemed a weak measure, particularly when compared with the earlier bills the reformers had presented.[41] But it had the decisive merit of conciliating the opposition sufficiently to secure its passage (not entirely unscathed) through the Lords.[42] By allowing the magistrates to continue to perform the duties of visitors in the provinces, it avoided giving offence to the country gentry; the provision exempting the county asylums, which were run by the local magistrates, had the same effect; while the parallel exemp-

39. S.W. Nicoll's *An Enquiry into the Present State of Visitation, in Asylums for the Reception of the Insane* (London: Harvey and Darton, 1828), published with a view to influencing the pending legislation, had provided a forceful restatement of the earlier arguments for the establishment of a full-time national inspectorate.
40. *Cf.* Nicholas Hervey, 'A Slavish Bowing Down', p. 101.
41. The Act did embody a number of new provisions, however. The commission could, for the first time, revoke or refuse licenses; a resident medical officer was now required for asylums with more than 100 patients; changes were made in the certification process, and some minimal requirements instituted concerning returns of patients and records of their treatment; and finally, the Evangelicals in the House of Lords imposed a requirement that divine service should be performed in every licensed house each Sunday.
42. Once again, both the Royal College of Physicians and a number of prominent medical madhouse keepers campaigned vigorously against the legislation, contending that it infringed on professional prerogatives, illegitimately interfered with private contracts between physicians and patients' families, and threatened to undermine mad-doctors' therapeutic efforts. For documentation of some of the behind the scenes lobbying, see the diaries of E.T. Monro, entries for 17, 25 March, 3, 12, 29 April, 14, 16, 17 May, 1828, cited in N. Hervey, 'A Slavish Bowing Down', pp. 101, 120.

tion of the charity hospitals circumvented the possibility that the powerful lobby formed by their aristocratic boards of governors would be ranged against the bill; and the provisions for the Metropolitan Commissioners to be amateurs stilled fears that the measure would produce a further increase in the powers of the executive.

Some of the most active reformers felt that the compromises had gone too far. S.W. Nicoll, the Recorder of York, who had been a close ally of Higgins in exposing the abuses at the York Asylum, was convinced the new Commission would be unwieldy and its impact too geographically limited. Despairingly, he commented that it offered 'no new stimulus, no new motive, no new intelligence . . .'.[43] In the long run, however, the Metropolitan Commission proved to be an extremely effective pressure group on behalf of the expansion of its own activities, and for the establishment of the full-time national inspectorate which the reformers had long sought.[44] All the leading members of the Parliamentary group of lunacy reformers initially secured appointments to the Commission – eleven of the fifteen Commissioners were MPs or former MPs, and their ranks included Robert Gordon, Anthony Ashley Cooper (later Lord Shaftesbury), and Lord Granville Somerset.[45] Such a small and politically influential group was well placed to use their reports as propaganda for their own solution to the problems of insanity and to produce evidence which would convince others of the correctness of their case. Moreover, they could confidently anticipate that while they enforced the provisions of the 1828 Act relatively vigorously, and thus eliminated at least the grosser forms of abuse, a majority of the provincial magistrates would continue to ignore or laxly carry out their responsibilities. Given time, the opponents of the centralized system could be relied upon to produce evidence which would damn their cause.

Entering upon their duties in 1828, the Metropolitan Commissioners

43. S.W. Nicoll, *An Enquiry Into the Present State of Visitation*, pp. 79, 84, 88–9.
44. For somewhat contrasting and more detailed recent assessments of the Metropolitan Commission's impact, see D.J. Mellett, 'Bureaucracy and Mental Illness: The Commissioners in Lunacy 1845–90', *Medical History* 25, 1981, pp. 223–4 (inclined to be sceptical of the Commission's impact); and N. Hervey, 'A Slavish Bowing Down', pp. 101–3, and *idem.*, 'The Lunacy Commission 1845–60, with Special Reference to the Implementation of Policy in Kent and Surrey, unpublished Ph.D. dissertation, Bristol University, 1987, pp. 70–114 (suggesting it played a more substantial role). My own review of the evidence corresponds more closely with Hervey's account.
45. Not surprisingly, many of the parliamentarians were too busy to participate in the routine work of inspection. In 1832, therefore, the Lord Chancellor, Lord Brougham, introduced legislation to cut back on the number of lay commissioners, and to add, for the first time, two barristers as legal commissioners. The first occupants of these positions, Mylne and Procter, played a vital role in systematizing the Commission's work, and in maximizing its effectiveness. It was a combination of the legal and lay commissioners (and not the medical commissioners) that made the Metropolitan Commission a powerful force for further change. See the discussions in N. Hervey, 'A Slavish Bowing Down', pp. 101–3; and *idem.*, 'The Lunacy Commission 1845–60', pp. 70–1.

found buildings that were dirty and ill-ventilated; inadequate numbers of attendants, with the consequence that mechanical restraint was widely employed; and patients left to vegetate, provided with neither occupations nor amusements. Establishments which took private patients were held to suffer less from these defects, but were 'nevertheless less satisfactory than might be hoped'.[46] By 1830, however, they were (somewhat implausibly) claiming that the more obvious abuses exposed by the 1827 Select Committee were in the process of disappearing. Following an inspection of Warburton's by now notorious White House, they reported that they were 'much gratified with the general condition of the House. . . . Mr Warburton has devoted much pains to the improvement of this establishment and the result is highly creditable'.[47] And they used their subsequent reports to drive home their claims about the value of thorough inspection. In 1836, for example, they asserted that 'in several respects, the improvement [in metropolitan madhouses] still continues to be progressive: among other points may be mentioned with praise the increased attention of the superintendents to cleanliness and ventilation, to classification of patients in wards, and to providing them with occupation and amusement . . . the amelioration has been marked and uniform'.[48] They had, they concluded, 'great reason to be satisfied with the improvements which have taken place in all the asylums within their jurisdiction'.[49]

On the other hand, evidence was readily forthcoming of instances where inspection in the provinces was cursory in the extreme. By 1838, the Commissioners were complaining that in the provinces, 'the salutary provisions of the Act by which it is required that all houses . . . shall be annually licensed and periodically visited and report by the Visiting Magistrates, have been in great degree neglected or violated . . .'.[50] As Conolly had predicted, the local justices proved to be reluctant to inquire into the conduct of a madhouse proprietor who was doubtless 'a most respectable man, a neighbour, a friend perhaps, and the asylum is his fortune; to depreciate it, or to cast doubt upon a case where he has none, may be to ruin him; and the sense of duty in an honorary Visitor could hardly lead him to run such a risk'.[51]

In 1839, a Select Committee of the House of Commons, set up in response to a petition from the Herefordshire magistrates for an inquiry into conditions at the Hereford Lunatic Asylum, produced evidence of widespread administrative irregularities and ignorance of the law on the part of both local magistrates and the proprietor of the asylum, defects in

46. Metropolitan Commissioners in Lunacy, *Report for 1830*, p. 4.
47. *Ibid.*
48. Metropolitan Commissioners in Lunacy, *Report for 1836*, p. 1.
49. *Ibid.*, p. 3.
50. Metropolitan Commissioners in Lunacy, *Report for 1838*, p. 6.
51. John Conolly, *An Inquiry Concerning the Indications of Insanity. With suggestions for the better protection and care of the insane*, London: Taylor, 1830, p. 6.

inspection which were presumed to have contributed to the neglect and ill-treatment of the patients.[52] In their report for the year, the Metropolitan Commissioners drew attention to these findings, as providing independent confirmation of the deficiencies of provincial inspection. By contrast, in successive reports, they made a point of emphasizing their own growing volume of work, and hence their need for more staff and more office space.[53]

4. *The Elaboration of a Pro-Institutional Ideology*

The Commissioners were not alone in seeking to revise and extend the reforms of 1828. The more respectable asylum doctors, who were attempting to monopolize the mad-business and to transform it into an arena for professional practice, were consequently interested in excluding the more disreputable elements engaging in the madhouse trade. The members of this proto-profession produced large numbers of books and pamphlets which served to bolster their claims to possess a specific expertise in the treatment of the insane; but many of these writers were also concerned 'to condense, in a plain, practical, and still popular form, the results of observation in the treatment of insanity, for the specific purpose of demanding from the public an amelioration of the condition of the insane'.[54] Over the next two decades, this emerging class of professional asylum administrators, acting in concert with a number of lay reformers, developed an increasingly elaborate account of the merits and advantages of the asylum as a response to insanity. Emphasis was placed on the contrast between conditions in traditional madhouses and the possibilities of the new approach based on moral treatment – both as providing more humane care than would otherwise have been possible, and as producing cures in a large number of cases. The growing volume of literature discussing what an asylum could do and what it should be like was matched by a growing optimism about the likely results of the new system. More and more the asylum was presented as a technical, objective, scientific response to the patient's condition, an environment which provided the best possible conditions for his recovery.

In the absence of public asylums, many pauper lunatics who were 'not so violent as to be perfectly unmanageable' were frequently abandoned, and left to 'linger out their existence in a workhouse'.[55] Here, the reformers objected, 'they are under the care of persons totally and entirely ignorant of the proper treatment of lunatics . . . the rooms in which they are kept

52. See House of Commons, *Report from the Select Committee on Hereford Lunatic Asylum, with Minutes of Evidence*, 1839.
53. See Metropolitan Commissioners in Lunacy, *Reports for 1838, 1839, 1840.*
54. W.A.F. Browne, *What Asylums Were, Are, and Ought to Be*, Edinburgh: Black, 1837, p. 1.
55. W.C. Ellis, *Letter to Thomas Thompson*, p. 10.

are ill-adapted to the confinement of such persons, and . . . from those causes, those unfortunate persons have been constantly confined in strait waistcoats, frequently kept in bed night and day.'[56] But though the maniac was here reduced to the status of an inconvenient object, whose condition required no more than that minimal efforts be made to ensure his physical survival, yet his presence was still a burden upon the workhouse administrators, and on the lives of the sane inmates. If nothing worse, the latter were forced to tolerate the disruptions and unpleasantness brought about by the lunatics' refusal to conform to ordinary social conventions, so that under even the most brutal and unfeeling management, 'lunatics in workhouses' unavoidably remained 'an extreme annoyance to the other inhabitants of those houses'.[57]

From this perspective, 'the propriety and necessity of a separate establishment for the care of lunaticks, must be obvious' to even the meanest intelligence.[58] The mental hospital would be an asylum in a dual sense: to the workhouse, it would afford relief from the disorder always at least latent in the presence of madmen; and for the maniac himself, it would provide a sanctuary, a refuge from the world with which he could no longer cope. Here he would be cared for by those who had a greater practical experience in the management of insanity, in surroundings specifically designed to avoid the structural deficiencies which, in the workhouse, made harsh treatment of the lunatic almost inescapable.

The promise of the specialized institution lay in its ability to create a forgiving environment in which humane care on a mass basis was possible; and the probability that the efforts of those running the asylums would restore a significant number of lunatics to sanity. In the aftermath of the publicity moral treatment had received, even those who did not entirely share the reformers' optimistic faith in the virtues of the asylum found it hard to resist the notion that the grim custodial routines by which the workhouses accommodated itself to the lunatic could be transformed. And since it was assumed that the insane in workhouses were only there because they lacked either the personal resources or the sort of supportive kinship network they needed to survive outside an institution, to advocate the continued segregation of such derelicts behind the walls of a different institution was scarcely controversial. If one could overlook the powerful deterrent factor of the cost of building and maintaining asylums (as parish officials were reluctant and reformers were prone to do),[59] then on most

56. Select Committee, *Report*, 1815, p. 11, evidence of Edward Wakefield.
57. *Ibid.*
58. St. Luke's Asylum, *Annual Report*, 1830, p. 3.
59. For instance, even after the 1827 Parliamentary inquiry had revealed that conditions in private madhouses in Middlesex were, if anything, worse than they had been in 1815, the overwhelming majority of petitions from parishes received by the magistrates' committee charged with erecting a county asylum urged rejection of the plan (twelve against versus only one in favour). Similarly, in Sussex, even after the passage of the 1845 Act which made provision of a county asylum compulsory, the opposition

other grounds it was plausible (and probably correct) to assert that at least those lunatics who had formerly starved and rotted in workhouse cellars would be better off in asylums.

But the reformers were not content just to bring these benefits to those who had hitherto been confined in sub-standard institutions. The asylum possessed advantages which even those whose families were still willing and able to care for them should share in. And the reformers and asylum doctors were determined that they should do so.

The idea of confining the sick or helpless members of one's family in an institution was not yet a popular one, particularly among the more respectable elements of society. Even the poorest families amongst the working classes made strenuous efforts to avoid bringing the disgrace of the workhouse upon themselves. Similarly, the hospitals of the period still tended to be patronized by a clientele composed of the indigent sick and the friendless traveller far from home. The enormous mortality rates, uncompensated for by greatly increased chances of recovery (to say nothing of the lingering stigma of their charity origins), gave them the public reputation of being little better than charnel houses, vectors of disease and death to be avoided by all who had the means to do so.

As for the deranged, the evils of the *ancien régime* madhouse had been widely advertised by the reformers themselves. The picture that had emerged from successive parliamentary inquiries and from the books and pamphlets produced by those agitating for lunacy reform incorporated a mixture of sex, madness, maltreatment, and murder in a fashion calculated at once to titillate and to repel. With male and female flung together without even the pretence of classification and supervision, the public was invited to imagine the 'decorum and purity of the intercourse which ensued,' bearing in mind that 'the beings thus having uninterrupted access to each other were irrational, acting under the impulse of ungovernable passions, and unrestrained, perhaps by the sacred obligations of religion, and certainly unmindful of the conventional check of public opinion'.[60] Meanwhile, even those madwomen who retained some remnants of innate feminine purity and modesty were not safe, their bodies being at the disposal of the lascivious ruffians who served as madhouse attendants. The deplorable consequences of a system in which the mad were '[r]egarded as wild beasts, [and] all maniacs were indiscriminately treated as such'[61] extended in every direction: patients bled and drugged into insensibility; their public display, like 'the animals in a menagerie'; unregarded deaths from botched force-feeding and the brutality of uncaring attendants; the

at the parish level was so determined and well organized that, despite continuous pressure from the Lunacy Commissioners, no county asylum was built until 1859. (The memorials opposing asylum construction, and the correspondence with the Lunacy Commission, are in the County Record Office at Lewes.)

60. W.A.F. Browne, *What Asylums Were, Are, and Ought to Be*, p. 128.
61. *Ibid.*, p. 101.

corrupt confinement of the sane, amidst the shrieks and ravings of the mad; and the remarkable and ingenious array of 'bolts, bars, chains, muffs, collars, and strait-jackets' madhouse keepers had devised to coerce a measure of order from recalcitrant human raw materials.

In such a context, it would not be difficult to conclude that asylums were 'rather . . . places for the concentration and aggravation, than for the relief of disease';[62] and the contention that everyone becoming insane ought to be promptly removed to an institution had an obviously paradoxical air.[63] So far from embracing the idea of institutionalization, a family concerned with the welfare of a member afflicted by mental disturbance, periodically regaled with evidence which appeared to confirm their worst gothic nightmares about what transpired behind the high walls and barred windows of the madhouse, must surely conclude that 'no person who can keep such a sufferer out, ought to place him in an asylum . . .'.[64]

But if asylum superintendents and their reformist mentors were to obtain a population consisting of much more than just chronic pauper derelicts, if the asylum was to amount to more than just a convenient dumping ground for confining the most troublesome flotsam and jetsam of society, then families who could exercise some choice in the matter had somehow to be convinced that the institution should be the place of first rather than last resort. Unless this effort succeeded, asylums would surely remain starved of funds. Moreover, without a significant proportion of upper-class patients, the newly emerging psychiatric profession could look forward to no more than a dubious status as a barely legitimate branch of medicine. For close and unremitting contact with the stigmatized and powerless carries its own peculiar reward – a share of their stigma and marginality.[65]

From a variety of perspectives, then, the construction of a more benign and salubrious image of the asylum, and the insistence on the potential curability of insanity when properly treated were vital components of the reformers' efforts to reshape attitudes towards madness and the mad, and

62. *Ibid.*, pp. 133, 107.
63. The problem is quite openly recognized and discussed by Andrew Combe, writing to the author of a pamphlet attacking conditions in contemporary asylums: 'Your work will strengthen the prejudice against asylums as moral lazar-houses . . . [by reinforcing] the popular notion of the cruel treatment of lunatics, and the great aversion thence arising either to have the patients removed to an asylum, or even to admit that insanity really exists.' The public must rather be 'led to regard asylums as infirmaries for the cure and kind treatment of that disease. . . .' Letter dated 13 July 1840, reprinted in George Combe, *The Life and Correspondence of Andrew Combe, M.D.*, Edinburgh: Maclachlan and Stewart, 1850, pp. 376–7.
64. Anon., *On the Present State of Lunatic Asylums*, London: Drury, 1839, p. 11. *Cf.* also W.A.F. Browne, *What Asylums Were, Are, and Ought to Be*, pp. 172, 177.
65. Compare J.C. Bucknill's complaint that 'the public extends its unreasonable antipathy to the insane, to all those who are connected with insanity; even to those who wrestle with the great evil, and, to the best of their ability, hold it down . . .'. 'Presidential Address', *Journal of Mental Science* 7, 1860, p. 6.

lay at the heart of their attempts to overcome the political and economic obstacles to lunacy reform. Yet in the light of the reformers' own findings, the conclusion that institutional care was preferable to even the best and most solicitous domestic arrangements was by no means self-evident. Hence it required elaborate ideological justification. Only by emphasizing the expertise of those who ran the asylums and the positive benefits of asylum treatment could the institution's advocates make a presumptive case for extending those 'benefits' to those not compelled to use the asylum's services.

The emerging profession was not above implying that any madman, even an apparently placid and harmless case of dementia, was capable of sudden and unprovoked acts of violence, which were peculiarly liable to be directed against members of his immediate family. This element of unpredictability carried with it the clear implication that 'both for their own safety, and that of others, it is necessary that they should be placed in a state of confinement, differing, of course, in degree as the symptoms are violent'.[66] But for all its functionality, the traditional argument, based on the threat the lunatic might pose to the security of the community, was more readily used to justify custody than efforts at cure; so that asylum superintendents were reluctant to employ it as their major argument for committal. Instead, they preferred to develop an alternative thesis, stressing that the management of insanity 'is an art of itself' and that in its successful treatment, 'the requisite means and advantages can rarely, if ever, be united in the private habitations even of the opulent'.[67]

The much greater experience of asylum personnel with the shapes and forms of mental disturbance was reflected in the ease and skill with which they managed and handled their patients. By contrast, the deranged frequently suffered from the well-meaning but misconceived interventions of devoted relatives. Not surprisingly, then, 'they submit more patiently to discipline from strangers, who are experienced in their treatment, than from relatives and dependents, who are timid, unskilled, and frequently the objects of irritation.'[68] Architecturally, too, '[a] private dwelling is ill-adapted to the wants and requirements of such an unfortunate being',[69] since in addition to there being a specific expertise attached to treating the insane, there were also buildings which were particularly adapted and suited to the condition of insanity. Only a purpose-built asylum could provide the requisite conditions for 'moral and physical management . . .

66. W.C. Ellis, *Letter to Thomas Thompson*, p. 8. See also John Parkin, *On the Medical and Moral Treatment of Insanity*, London: Martin, [1843?]; and Robert Gardiner Hill, *A Lecture on the Management of Lunatic Asylums and the Treatment of the Insane*, London: Simpkin, Marshall, 1839, p. 6.
67. Reverend J.T. Becher, *An Address to the Public on the General Lunatic Asylum near Nottingham*, Newark: 1811, pp. iv–v.
68. [Edward Long Fox], *Brislington House: An Asylum for Lunatics*, Bristol: n.p., 1806, p. 1.
69. R.G. Hill, *Lecture on the Management of Lunatic Asylums*, p. 6.

as well as the means of preventing personal injury and inconvenience to the Patient'.[70]

Besides the fact that it lacked most of the basic necessities for the effective treatment of mental illness, the home suffered from a fatal flaw, one which made it the least suitable place of all to keep a lunatic. Quite simply, it was the environment which had nurtured the disturbance in the first place. Lunatics ought not to be treated at home, 'not only on account of the distress and confusion they there produce, but because there circumstances that excite a maniacal paroxysm frequently exist'.[71]

If any families were concerned with the welfare of their insane members, it was surely those who, rather than simply abdicating all responsibility by handing them over to strangers, were prepared to suffer the inconvenience produced by keeping such people at home. But if the asylum doctors were to be believed, such generous and self-sacrificing instincts were fraught with potentially disastrous consequences for the patient himself. After all, 'experience . . . proves that few are cured at home, but many more when removed'.[72] The overwhelming consensus of the experts in the field testified to

> the improbability (I had almost said moral impossibility) of an insane person's regaining the use of his reason, except by removing him early to some Institution for that purpose. If such a result is ever to be obtained without the adoption of this plan, it is either a very rare occurrence indeed, or it has ensued from a change of residence, of scene, and of persons around, combined with a mode of treatment in some measure resembling that which can be adopted fully only in a Building constructed for the purpose.[73]

The need for reassurance that 'those we exile to madhouses are receiving treatment, not punishment'[74] seems to have emerged early in the history of the asylum, and to have been offered and accepted on the basis

70. [E.L. Fox], *Brislington House*, p. 1.
71. [E.L. Fox], *Brislington House*, p. 1. See also *The State of Lincoln Hospital in 1837: 13th Annual Report*.
72. *Westminster Review* 18, 1833, p. 134. This was a safe generalization, since nobody possessed (or was likely to possess) the information necessary to disprove it.
73. R.G. Hill, *Lecture on the Management of Lunatic Asylums*, pp. 4–5. See also Maximilian Jacobi, *On the Construction and Management of Hospitals for the Insane* (translated by John Kitching), London: Churchill, 1841, p. 7. Compare also George Man Burrows (*Commentaries on the Causes, Forms, Symptoms, and Treatment of . . . Insanity*, London: Underwood, 1828, p. 696): 'There is no general maxim in the treatment of insanity wherein medical practitioners, ancient or modern, foreign or domestic, are so unanimous, as that of separating the patient from all customary associations, his family, and his home. . . . Few of the medical profession require arguments or proof to convince them of the great utility of separation. When the friends of any insane person have a doubt, and have not confidence in the advice or probity of their physician, they should consult the works of authors who have treated of insanity, and who can have no interest in the question.'
74. Erving Goffman, *Asylums*, Garden City, New York: Doubleday, 1961, p. 369.

of very little evidence. Perhaps this should not surprise us: after all, it serves important functions for both the madhouse keepers and their true clients, the 'patient's' family. For the former, it provides the necessary material from which the self-image of expertise can be constructed. For the latter, it eases the guilt which must inevitably be attached to the prospect of confining a loved one in an institution which has never managed to obtain a very salubrious reputation. Accordingly, both groups could find comfort in the assurance that 'the treatment of insanity is now so well understood, and the requisites for forming a perfect asylum, or hospital, for the reception of insane patients so fully ascertained by experience that they admit of very little discussion'.[75]

5. *The Asylum's Critics*

By and large, the reformers and the asylum doctors were extremely effective in proselytizing the asylum's virtues and the need to insulate the insane from the world. From time to time, the wisdom accumulated by the experts was condensed and summarized for popular consumption in one or another of the leading reviews of the period.[76] And laymen could always be impressed by references to a veritable pantheon of names famous for their humanity and/or their skill in treating mental disorder – Cullen, Tuke, Pinel, Esquirol – all of whom were staunch advocates of the seclusion of the insane.

Occasionally, though, a few isolated figures refused to bend to the weight of professional and informed opinion, and raised their voices against the tendency to incarcerate all those labelled as mad. For Andrew Combe, the judgement that someone was insane was quite distinct from the question of whether confinement in an institution was appropriate, 'and this distinction ought never for a moment to be forgotten'.[77] Granted there were certain cases – where the home was a source of 'incessant irritation' or where 'morbid associations have . . . connected themselves inseparably with his own family and friends' – in which 'the preference ought to be given to a well-regulated asylum . . .'.[78] But '[b]ecause confinement is beneficial to the recovery of one class of patients, it has been supposed that it must be equally advantageous for the recovery of all; and

75. Andrew Halliday, *A Letter to the Magistrates of Middlesex on Erecting an Asylum for Pauper Lunatics*, London: Brettell, 1826, p. 13.
76. Compare 'Esquirol and the Treatment of the Insane', *Westminster Review* 18, 1833, pp. 129–38; 'Lunacy', *Westminster Review* 37, 1842, pp. 305–21; 'Lunatic Asylums', *Westminster Review* 43, 1845, pp. 162–92; [W.H. Fitton], 'Lunatic Asylums', *Edinburgh Review* 28, 1817, pp. 431–71; 'Insanity', *Quarterly Review* 24, 1821, pp. 169–94.
77. Andrew Combe, *Observations on Mental Derangement*, Edinburgh: Anderson, 1831, p. 345.
78. *Ibid.*, pp. 346–7, 349.

to pronounce a person insane, and to send him to a madhouse, have thus come to be considered as almost the same thing . . .' – a practice he dismissed as 'an utter and injurious absurdity'.[79] On the contrary, 'Many who have been hurried into premature confinement, have, by that very act, been fitted for remaining tenants for life of the cells to which heedless rashness first consigned them.'[80] In approaching the question of what to do with the insane, the first requisite was thus 'to get rid of the hitherto inseparable association between insanity and the madhouse'.[81]

Thomas Bakewell, perhaps the best-known non-medical madhouse keeper of the early nineteenth century, derived a substantial income from his small private asylum at Spring Vale. Consequently, one is tempted to dismiss his opposition to plans for a system of publicly-funded asylums as anything but disinterested. The burden of his complaint, however, echoed the concerns of less partial observers:

> Large Public Asylums for the Insane, are certainly wrong, upon system; for nothing can be more calculated to prevent recovery, from a state of Insanity, than the horrors of a large Mad House, close confinement, and a state of idleness in the company of incurable Lunatics.[82]

Others, indeed, were still more hostile to institutionalization. George Nesse Hill, author of one of the most well-known early nineteenth-century texts on the management and restoration of lunatics, insisted that 'congregating insane people together in the promiscuous way which has too long obtained is an evil of the most pernicious tendency'.[83] The separation from the sane influences which surrounded the mad in the outside world exacerbated their problems, and the unfortunate inmates of madhouses tended to feed off one another's delusions. Consequently, both 'public charities and private asylums stand opposed to all rational plans of speedy and permanent cure of insanity, and from their very nature are the most unfavourable situation in which . . . lunatics . . . can be placed'.[84] John Reid's verdict on asylums was couched in yet harsher terms: decrying 'our medical prisons . . . our *slaughter houses* for the destruction and mutilation of the human mind',[85] he alleged that they fixed the very delusions they claimed to remove. Indeed, 'Many of the depots for the captivity of intellectual invalids may be regarded only as nurseries for and manufactories of madness; magazines or reservoirs of lunacy, from which is

79. *Ibid.*, p. 346.
80. *Ibid.*, p. 348.
81. *Ibid.*, p. 343.
82. Thomas Bakewell, *A Letter . . . to . . . the Select Committee of the House of Commons*, p. 9.
83. G.N. Hill, *An Essay on the Prevention and Cure of Insanity*, London: Longman, Hurst, Rees, Orme, and Brown, 1814, p. 222.
84. *Ibid.*, p. 220.
85. John Reid, *Essays on Insanity, Hypochondriacal and Other Nervous Affections*, London: Longman, Hurst, Rees, Orme, and Brown, 1816, p. 206, emphasis in the original.

issued, from time to time, a sufficient supply for perpetuating and extending this formidable disease.'[86]

John Conolly, newly installed as Professor of the Nature and Treatment of Diseases at the fledgling University of London,[87] pointed out that it was seldom hard to isolate particular features of someone's behavior which would lend credence to the idea that he was crazy. To send such a person to an asylum was practically to ensure that the designation would stick: 'Once confined, the very confinement is admitted as the strongest of all proofs that a man must be mad.' Hence to create more institutions would guarantee the discovery of more madmen to fill them. Already, 'the crowd of most of our asylums is made up of odd but harmless individuals, not much more absurd than numbers who are at large'.[88] Moreover, even for those who were manifestly mad, the asylum was more likely to prove harmful than otherwise. After all, an anonymous fellow critic argued,

> what would be the consequence, if we were to take a sane person, who had been accustomed to enjoy society, and . . . were to lock him up in a small house with a keeper for his only associate, and no place for exercise but a miserable garden? We should certainly not look for any improvement to his moral and intellectual condition. Can we then reasonably expect that *a treatment which would be injurious to a sane mind, should tend to restore a diseased one?*[89]

The most vital deficiencies of the asylum as a curative institution were thus inherent in its very structure. No matter that asylum directors were 'men of great intelligence and humanity'; no matter that

> they may point to the spaciousness of their grounds, to the variety of occupations and amusements prepared for their patients; to the excellence of their food and the convenience of their lodgings; and urge that as little restraint is employed as is compatible with their safety;

86. *Ibid.*, p. 205. Such statements echoed a scepticism about the merits of institutions that can also be traced among some eighteenth-century writers (when, in general, the value of asylum treatment was portrayed in far more circumscribed terms). Andrew Harper, for instance, claimed that asylum care was wholly counter-productive, 'big with ignorance and absurdity. [Confinement,] 'tis true, may answer the purpose of private interest, and domestic convenience, but at the same time it destroys all the obligations of humanity, robs the sufferer of every advantage, and deprives him of all the favourable circumstances which might tend to his recovery . . . confinement never fails to aggravate the disease. A state of coercion is a state of torture from which the mind, under any circumstances, revolts.' Andrew Harper, *A Treatise on the Real Cause and Cure of Insanity*, London: Stalker and Walter, 1789, pp. 59–61.
87. An appointment that lasted a scant two years, following which he (temporarily) sank back into provincial obscurity. See my *Social Order/Mental Disorder: Anglo-American Psychiatry in Historical Perspective*, Berkeley: University of California Press/London: Routledge, 1989, pp. 169–85.
88. John Conolly, *An Inquiry Concerning the Indications of Insanity*, London: Taylor, 1830, pp. 4–5, 17.
89. Anon., *On the Present State of Lunatic Asylums*, p. 39, emphasis in the original.

> but the fault of the association of lunatics with each other, and the infrequency of any communication between the patient and persons of sound mind mars the whole design.[90]

For those forced to remain within the walls of an asylum, 'the effect of living constantly among mad men and women is a loss of all sensibility and self-respect or care; or, not infrequently, a perverse pleasure in adding to the confusion and diversifying the eccentricity of those about them. . . . In both cases the disease grows inveterate.'[91] Such a pathological environment thus encourages the very behaviours which are then used to justify its existence: 'Paroxysms of violence alternate with fits of sullenness; both are considered further proofs of the hopelessness of the case.'[92] The peculiar routines of the asylum are quite unlike those of the outside world. Nevertheless, some inmates manage to adjust to them and it is one of the ironies of institutional existence that those who manage this transition most effectively are at the same time reducing their ability to function in the outside world. 'After many hopeless years, such patients become so accustomed to the routine of the house, as to be mere children; and are content to remain there as they commonly do, until they die.'[93] Equally pathetic were the 'numerous examples . . . in which it was evident that although the patients were not yet sufficiently recovered to be restored to their families without superintendence, a continued residence in the asylum was gradually ruining the body and the mind'.[94]

If some of these critics seemed to argue for the total abolition of lunatic asylums, others, as I have indicated, were more circumspect. Andrew Combe, while urging that many of those who could not be cared for in their own homes might nevertheless be better placed outside an asylum (in a physician's household for instance, or boarded with an attendant 'hired for the purpose'), still conceded that in certain cases, 'the preference ought to be given to a well-regulated asylum, from the superior means which such an institution affords for enforcing regularity and order, and the various other measures conducive to the safety, comfort, and recovery of the patient'.[95] John Conolly, too, urged that in certain circumstances, 'public lunatic asylums' must be regarded as 'unavoidable evils'.[96] They were, however, totally unsuitable for most of the insane: curable cases ought never to be shut up in an asylum; nor should the large class of the harmlessly demented; indeed, '[n]o patient should be confined in a Lunatic

90. J. Conolly, *An Inquiry Concerning the Indications of Insanity*, p. 31.
91. *Ibid.*, p. 22.
92. *Ibid.*
93. *Ibid.*, p. 21.
94. *Ibid.*, p. 20.
95. A. Combe, *Observations on Mental Derangement*, p. 349.
96. J. Conolly, *An Inquiry Concerning the Indications of Insanity*, p. 7.

Asylum, except on the particular representation of the relatives or friends, that he could not have proper care and attention out of it.'[97]

The clear implication of these arguments was that the whole effort to reform the treatment of the insane by their routine isolation in hospitals was a venture which was misconceived from the start.[98] Resting their case on a faulty and incomplete analysis of what was wrong with traditional madhouses, the reformers were committing themselves to 'solutions' which, in the nature of things, could not produce a lasting improvement in the condition of the insane.[99] Yet, despite the force and relevance of the critique, it drew no intellectually adequate response from the asylum's proponents.[100] For all the impact these words had, they might as well have never been uttered. It is not just that they had no influence on social policy, or that they were met by counterarguments which seemed plausible at the time. Rather, their fate was to be greeted by silence, to be consigned to oblivion.

Quite clearly, the critics suffered from their lack of organization and numbers, which inhibited their ability to get their opponents to take them seriously. They were, after all, attempting to overturn the conventional wisdom of the overwhelming majority of experts in the field. Those experts had a considerable intellectual investment in the asylum; over many years, they had spent considerable time and effort trying to educate the public in the necessity of building asylums. Indeed, they had already persuaded the community to make a sizeable investment of material

97. *Ibid.*, pp. 29, 30, 48, 483. I would argue that even a program of building asylums based on these minimalist assumptions would in all probability have led to the ultimate dominance of the institutional response. What Combe and Conolly failed to realize is that what a family perceives as intolerable behaviour on the part of one of its members, up with which it will not/cannot put, is itself a socially variable matter. The availability of a culturally legitimate alternative to keeping him/her in the family itself tends to reduce family tolerance (to put it another way, to expand the notion of the intolerable) to a degree which varies with how grandiose and well accepted the institution's helping claims are. For further discussion of this point, see chapter 7 below.

98. Compare Sir Robert Peel: 'There could not be a question that, unless asylums for pauper lunatics were well-conducted, they would be a curse rather than a blessing; and it would be infinitely better to have none at all, than such as would only offer temptations to send unfortunate creatures to them. There were many cases in which the patient was merely troublesome, and it was much better that such as these should be abroad, it being preferable to leave them in the custody of their relatives, than lock them up in madhouses.' Quoted in Joseph Mortimer Granville, *The Care and Cure of the Insane*, Vol. 1, London: Hardwicke and Bogue, 1877, p. 87.

99. Thomas Bakewell, himself a third generation madhouse keeper, voiced extreme scepticism at the reformers' proposal for a network of state-run asylums, suggesting that the unintended consequences of its enactment would prove worse than the defects it was supposed to remedy: 'What is the system at best but a scheme to organize by law indiscriminate coercion, from which there is no appeal, under the ostensible plea of humanity?' *A Letter . . . to . . . the Select Committee of the House of Commons*, p. 20.

100. For very qualified exceptions to this general contemporary neglect, see 'Esquirol on the Treatment of the Insane', *Westminster Review* 18, 1833, pp. 129–38; Maximilian Jacobi, *On the Construction and Management of Hospitals for the Insane*, London: Churchill, 1841.

resources in just such a program. These were not the sort of commitments which would be easily abandoned.

As I shall show later in this chapter, the proponents of institutionalization were by now so convinced of the superiority of a carefully supervised asylum system that they were prepared to overlook, or rationalize away, not merely plausible *arguments* that they were wrong, but also a mass of empirical evidence which indicated that the strategy which they had chosen would, from the outset, fall far short of the goals they had set for it. This blindness or tunnel vision derived not merely from the normal reluctance of most human beings to concede that they may be mistaken, but also from a specific trait which one must possess to be a successful reformer. It is plainly the case that awareness of the complexity of the world and the moral ambiguity of existing patterns of behavior is something which always threatens to paralyse action; so that all of us are forced to resort to some degree of cognitive simplification of the world, to take for granted the 'routine grounds of everyday action', if the world is not to overwhelm us. The reformer, though, the person who seeks to alter certain aspects of this socially constructed reality, must characteristically carry such cognitive simplification to an extreme. The ability to reduce the ambiguous to the certain, to order the world in an often grossly stereotypical fashion, is generally a necessary precondition if one is to be a successful moral crusader. If the reformer is to remain dedicated to his task in the face of the ridicule and opposition he is sure to encounter, he must possess abundant confidence in the validity of his chosen alternative; and if he is to convince others of the urgent necessity to change existing arrangements, he must provide convincing, and therefore one-sided, 'proof' of his contentions.[101]

Quite apart from the single-mindedness of the reformers, and their consequent lack of receptivity to counterarguments, certain structural factors made their solution more attractive to the governing classes than its competitor. It was all very well to suggest that the reformers' cure was worse than the disease, but what was the alternative? What were the likely implications of leaving the insane in the community?

There was no systematic evidence available to answer the latter question. Assuming that all those who would later find their way into an asylum and be labelled insane could have been readily identified while still in the community (a large assumption), an adequate assessment of how they would be treated would have been a complex and time-consuming

101. Henry Maudsley characteristically offered a harsher analysis: 'He who possesses great judgment and has his feelings completely under the control of his reason is not likely to be a great reformer in his lifetime . . . the reformer . . . is not very far removed from the monomaniac . . . [indeed,] the extreme earnestness and sincerity of a little insanity are almost essential to move a man to the unhopeful and laborious task of combating the heavy opposition of the existing state of things.' Henry Maudsley, 'Delusions', *Journal of Mental Science* 9, 1863, pp. 5–6, 8.

task. It was never attempted. The reformers, at least, were content to rely on a small number of what Goffman has called 'exemplary tales' to demonstrate their contention that leaving the insane in the community invited the grossest forms of abuse. Little or no effort was made to determine whether such stories were in fact representative. Instead, isolated reports of individual cases of maltreatment were elevated to the status of definitive proof of the barbarity of existing conditions.

It is, of course, likely that by the standards of the typical middle-class reformer, the living conditions of the pauper insane would seem appalling. On may doubt, though, whether the reasons for this lay simply in the tendency of the ignorant to maltreat the mad. Living conditions of even the sane members of the lower classes, particularly those crowded together in the new urban slums, were quite sufficient to provoke expressions of horror and disgust in those of their betters who came into contact with them. Where the sane were forced to live in squalor, disease, and misery, on the very edge of starvation, the treatment of a dependent and often troublesome group like the lunatics was unlikely to correspond with middle-class ideals.

This would remain true even if the lunatic fared no worse than other members of his or her community. Certainly, few of those concerned with the plight of the insane could contemplate with equanimity the prospect of leaving them in the sorts of conditions which commonly prevailed in the larger towns:

> It will scarcely appear credible, though it is precisely true, that persons of the lowest class do not put clean sheets on their beds three times a year; that even where no sheets are used, they never wash or scour their blankets or coverlets, nor renew them until they are no longer tenable; that curtains, if unfortunately there should be any, are never cleansed, but are suffered to continue in the same state until they fall to pieces; lastly, that from three to eight individuals of different ages often sleep in the same bed; there but one room and one bed for each family. . . . The room occupied is either a deep cellar, almost inaccessible to the light, and admitting of no change of air; or a garrett with a low roof and small windows, the passage to which is close, kept dark, and filled not only with bad air, but with putrid excremental effluvia from a vault at the bottom of the staircase . . . the cumbrous furniture or utensils of trade with which the apartments are clogged, prevent the salutary operations of a broom . . . and favour the accumulation of a heterogeneous filth.[102]

Millions of English men, women, and children were virtually living in shit. The immediate question seems to have been whether they weren't

102. R. Willan (1801), quoted in M.D. George, *London Life in the Eighteenth Century*, London: Penguin, 1965, p. 95.

> drowning in it. . . . Large numbers of people lived in cellars, below the level of the street and below the water line. Thus generations of human beings, out of whose lives the wealth of England was produced, were compelled to live in wealth's symbolic counterpart. And that substance which suffused their lives was also a virtual objectification of their social condition, their place in society: that was what they were.[103]

In such a situation, those who sought to improve the lot of the pauper insane, but who were dubious about the merits of the asylum, confronted a painful dilemma. They could scarcely dispute MacGill's claim that 'the circumstances of the great body of mankind are of such a nature as to render every attempt at recovering insane persons in their own houses extremely difficult, and generally hopeless'.[104] And if they balked at the idea of keeping lunatics in such surroundings, it was hard to see how they could avoid concluding that the asylum was better than the other option available, the workhouse.

What stood in the way of ameliorating the environment of the insane still at large? To improve the conditions of existence for lunatics living in the community would have entailed the provision of relatively generous pension or welfare payments to provide for their support, implying that the living standards of families with an insane member would have been raised above those of the working class generally. Moreover, under this system, the insane alone would have been beneficiaries of something approximating a modern social welfare system, whilst their sane brethren were subjected to the rigours of a Poor Law based on the principle of less eligibility. Such an approach would clearly have been administratively unworkable, not least because of the labile nature of lunacy itself, and the consequent ever-present possibility that given sufficient incentive (or rather desperation), the poorer classes would resort to feigning insanity.

In any event, a program of this sort had absolutely no political appeal to the upper classes as a whole. Engraved deeply into bourgeois consciousness in this period was an abhorrence of all forms of outdoor relief. On the one hand, this abhorrence reflected the lessons they had learned concerning the evils of such relief from the disastrous impact of the Speenhamland system – a system which had undermined the wage labour system while pauperizing, demoralizing, and degrading 'the people it was [allegedly] designed to succour'.[105] On the other hand, the determination to avoid the payment of relief to those in the community was at once reflected and strengthened on the ideological level by the hegemony of classical liberalism. For the logical consequence of that doctrine's insistence that

103. Steven Marcus, *Engels, Manchester, and the Working Class*, New York: Vintage, 1974, pp. 184–5.
104. Stevenson MacGill, *On Lunatic Asylums*, Glasgow: for the Glasgow Asylum Committee, 1810, p. 4.
105. Karl Polanyi, *The Great Transformation*, Boston: Beacon, 1957, p. 81.

each man was to be free to pursue his fortune and at the same time was to be responsible for his own success or failure, coupled with its dogmatic certainty that interference with the dictates of the free market could only be counterproductive in the long run (a proposition that could even be proved theoretically), was to render the whole notion of social protectionism – in any form – an anathema. Moreover, by the 1830s, the English ruling classes had developed

> a sense of precariousness about society. This was expressed in the form that there was a delicate balance between institutions and their operation, and the behaviour of the labouring classes. There was a feeling that any concession to idleness might bring about a rapid and cumulative deterioration in the labourer's attitude towards work. This produced a growing sensitivity towards the Poor Law[106]

– and towards anything else that, by lessening the dependence of the labouring classes on market forces, might weaken the social fabric of Victorian society.

These obstacles presented a virtually insurmountable barrier to the adoption of any alternative scheme. Only the asylum plan offered the advantage of allowing scope for the exercise of humanitarian impulses, without requiring any fundamental changes in the structure of society and while remaining consistent with the imperatives of the New Poor Law. Bowing to the inevitable, the anti-institutional mavericks fell into line. Within a few years, we find that Conolly became a leading and zealous advocate of county asylums for pauper lunatics.[107] He had been anticipated by one of his (anonymous) fellow critics, who, for all the evils he saw in the asylum (which was 'a prison' in which 'the want of all society, the absence of all amusement and employment, both of body and mind, must tend to *increase* rather than to relieve the morbid irritation of the brain'), thought from the outset that such establishments were perfectly satisfactory for paupers: 'As . . . the county or public asylums will soon become general . . . no fear need be entertained for the future fate and condition of pauper lunatics in this country.'[108] Even its staunchest opponents now conceded the asylum's inevitability.

6. *The Model Institution*

Given their basic commitment to the asylum as the solution to the problem of what to do with lunatics, the reformers laboured hard to translate

106. S.G. Checkland and E.O.A. Checkland (eds) 'Introduction' to *The Poor Law Report of 1834*, London: Penguin, 1974, pp. 20–1.
107. See John Conolly, *On the Construction and Government of Lunatic Asylums*, London: Churchill, 1847, and the discussion in Andrew Scull, 'John Conolly: A Victorian Psychiatric Career', pp. 162–212 in *Social Order/Mental Disorder*, Berkeley: University of California Press/London: Routledge, 1989.
108. Anon., *On the Present State of Lunatic Asylums*, pp. 16, 41, 52. Presumably, paupers' sensibilities were insufficiently refined to notice the defects of institutional confinement.

their ideals into reality. They tried to provide detailed blueprints of how an asylum should be organized and what its principal features ought to be. Throughout, it is quite clear that the dominant influence on their minds, and the model they sought perpetually to keep before them, was the York Retreat; and the writings of Samuel Tuke remained their constant guide.

The contrast with the traditional madhouse – the shit, straw, and stench, the beatings, intimidation, and rapes – could scarcely be more marked. So far from being 'a moral lazar house'[109] wherein the deranged were hidden and hope and humanity abandoned, the asylum was to be a home, where the patient was to be known and treated as an individual, where his mind was to be constantly stimulated and encouraged to return to its natural state. Mental patients required dedicated and unremitting care, which could not be administered on a mass basis, but rather must be flexible and adapted to the needs and progress of each case. Such a regime demanded kindness and an unusual degree of forbearance on the part of the staff. If this ideal were to be successfully realized, the attendants would have to be taught to keep constantly in mind the idea 'that the patient is really under the influence of a disease, which deprives him of responsibility, and frequently leads him into expressions and conduct the most opposite to his character and natural dispositions'. For this teaching to be successful, and since the attendant was the person who had the most extensive and intimate contact with the patient, attendants should be selected for their intelligence and upright moral character.[110]

Even these precautions would probably not suffice, though. They recognized that 'the business of an attendant, requires him to counteract some of the strongest principles of our common nature'.[111] Samuel Tuke himself had pointed out that the practice of moral treatment required a degree of altruism only present in those with a true vocation. After all, 'to consider them [the insane] at the same time both as brothers, and as mere automata; to applaud all they do right; and pity, without censuring, whatever they do wrong, requires such a habit of philosophical reflection, and Christian charity, as is certainly difficult to attain.'[112] It was not considered impossible, however. The influence of the superintendent could be a powerful force for good here. In a properly run asylum, the patients must be seen daily, sometimes hourly, by the man who had charge of the institution. By paying 'minute attention' to all aspects of the day-to-day conduct of the institution, by always setting, through his own example, a high standard for subordinates to emulate in their dealings with the inmates, he could foster the kind of intimate and benevolent familial environment in which acts of violence would become rare. Indeed, as the

109. W.A.F. Browne, *What Asylums Were, Are, and Ought to Be*, p. 213.
110. S. Tuke, *Description of the Retreat*, p. 175.
111. S. Tuke, *A Letter to Thomas Eddy of New York on Pauper Lunatic Asylums*, New York: Wood, 1815, p. 27.
112. S. Tuke, *Description of the Retreat*, p. 176.

autocratic guiding spirit of the whole curative apparatus, the superior moral and intellectual character of the medical superintendent was an essential precondition for success. From their own experience, though, the reformers knew that 'education and talent are but imperfect securities against the seductions of interest and indolence,' so they tried to provide a further barrier to history repeating itself: in the last analysis, they concluded, the best, even 'the only security for the good conduct of the attendants [and medical officers] is the most frequent inspection'.[113]

The emphasis on intimacy, on patients and staff alike being members of an extended 'family', was, at the outset at least, not confined to a mere rhetorical flourish. The propagandists for the asylum vigorously promoted this idea and sought to ensure that it would be carried into effect. Hence, above all else, they emphasized the virtues of small size. Without it, 'the attempt to introduce anything approaching to domestic comfort, is altogether futile; and it becomes necessary for the attendant to rule with an iron hand, to keep in order such a formidable body of malcontents'.[114] 'It is evident,' said Ellis,

> . . . that for the patients to have all the care they require, there should never be more than can, with comfort, be attended to; from 100 to 120, are as many as ought to be in any one house; where they are beyond that the individual cases cease to excite the attention they ought; and if once that is the case, not one half the good can be expected to result.[115]

Others thought that the number might be raised to 200, or even 250; but all the major authorities agreed that it should not rise beyond this point.[116]

The concern with small size, and with recreating the family in the institution, did not end here. An institution of fewer than a hundred could dissipate all the advantages which that gave it, if it neglected to classify the patients according to the severity of their disorder, and if the wards themselves grew too large. The ideal size was not more than ten people, and Tuke warned of the consequences of breaking that rule:

> During the last year, I had frequent occasion to visit two Institutions for the insane, in which very opposite plans . . . were adopted. In one, I frequently found upwards of thirty patients in a single apartment; in the other, the number in each room rarely, if ever, exceeded ten. Here, I generally found several of the inmates engaged in some useful or amusing employment. Every class seemed to form a little family; they observed each other's eccentricities with amusement or pity; they were

113. S. Tuke, *A Letter . . . on Pauper Lunatic Asylums*, pp. 27–8.
114. *Ibid.*, p. 15.
115. William Ellis, *A Treatise on the Nature, Symptoms, Causes, and Treatment of Insanity*, London: Holdsworth, 1838, p. 17.
116. *Cf.* M. Jacobi, *On the Construction and Management of Hospitals for the Insane*, p. 23; Metropolitan Commissioners in Lunacy, *1844 Report*, p. 23.

Fig. 12 An engraving by Dean, after Musgrave, of Chester County Asylum, one of the earliest county asylums, built under the permissive act of 1808 and opened in 1829, from Hemingway, *Chester* (*c.*1830). Already more 'institutional' than its supposed inspiration, the York Retreat, its relatively modest size nonetheless contrasts sharply with the vastness of asylums built only a decade or so later. Within a generation, such small-scale public asylums had vanished.

> interested in each other's welfare, and contracted attachments or aversions. In the large society, the difference of character was very striking. I could perceive no attachments, and very little observation of each other. In the midst of society, everyone seemed in solitude; conversation or amusement was rarely to be observed – employment never. Each individual appeared to be pursuing his own busy cogitations; pacing with restless step from one end of the enclosure to another, or lolling in slothful apathy on the benches. It was evident that society could not exist in such a crowd.[117]

Even the architecture and the physical setting of the asylum could make vital contributions to its success, concretely exemplifying the distance between the reformed institution and the traditional madhouse. A modern curative establishment should be sited where the patients could enjoy the benefits of fresh, bracing country air, and where there was an extensive and pleasing view of the surrounding countryside to divert the mind from its morbid fantasies. The building itself should emphasize as little as possible the idea of imprisonment or confinement. The insane were very sensitive to their surroundings, and it was not extravagance to design and

117. Samuel Tuke, *A Letter on Pauper Lunatic Asylums*, pp. 14–5.

build institutions which emphasized cheerfulness by being aesthetically pleasing. Indeed, spacious and attractive accommodations could make their own contributions to the inmates' 'moral training', and to replacing 'their morbid feelings . . . [with] healthy trains of thought'.[118] To further these goals, the buildings should avoid a dull uniformity and their internal arrangements should be such as to allow a maximum of organizational flexibility. Patients, for instance, ought to be able to change rooms in the course of the day to get a change of scenery.

For essentially the same reasons, provision ought always to be made for extensive grounds to be attached to an asylum. These would allow scope for recreation and harmless diversion, the kinds of mental and physical stimulation which would counter the tendency of insanity to degenerate into outright fatuity. 'The leading principle here is to prevent idleness, to preserve every power of mind and body constantly occupied, and never to allow it to flag or to retire upon itself.'[119] And one of the best ways of putting that principle into practice was to encourage the patients to employ themselves in some useful work. Attaching a farm to every hospital would provide admirable opportunities for the kind of regular employment which greatly helped to restore men's minds.

Classification, separation, and employment – all central to the implementation of moral treatment – had a still broader significance, providing the basis for a response to those critics who objected that asylums, by 'congregating insane people together in the most promiscuous way', constituted 'an evil of the most pernicious tendency . . .'.[120] Reassurance could be offered that the tranquil and the raving, the convalescent and the incurable, were carefully separated; treatment individualized and adapted to the idiosyncracies of the patients (an implicit and telling contrast with the indiscriminate mass-medication of the *ancien régime*); and interaction managed and controlled in carefully constructed communities of the mad. Classification also allowed asylum keepers to offer still another kind of reassurance: that no improper mingling of the social classes would be permitted. One might well imagine the 'unhappiness which would flow from bringing the ignorant and brutal into constant and compulsory contact with the enlightened and refined'.[121] Besides, 'The pauper could not appreciate, nor prize, nor derive benefit from the refinement and delicacies essential to the comfort, and instrumental in the recovery of the affluent. Most fortunately, [therefore,] this arrangement, which is called for by the usages of society, is found to correspond with those higher and less artificial distinctions which are dictated by philosophy.' Class distinctions finding support in Nature itself, the asylum superintendent could

118. W.A.F. Browne, *What Asylums Were, Are, and Ought to Be*, p. 191.
119. William Nisbet, *Two Letters to George Rose M.P. on the . . . State of the Madhouses*, London: 1815, p. 26.
120. G.N. Hill, *An Essay on Insanity*, p. 222.
121. W.A.F. Browne, *What Asylums Were, Are, and Ought to Be*, p. 201.

proceed with a clear conscience to make 'fitting preparations' with which to 'tempt the rich to have recourse to those measures from choice, which the poor have long pursued from necessity'.[122]

The advocates of the asylum, then, plainly saw that there were obstacles to the successful realization of their ideal. But they were infinitely remote from the view which saw the proposed cure as worse than the disease, or even as tending only to spread the disease. Rather they were confident that no difficulty was insurmountable, that all the problems they and others had identified could be overcome – indeed, that they already had the answers to most of them. Within the controlled confines of the institution, they announced that the paternal order Tuke and his successors had created would provide a practical demonstration that even the irrational and raving could be reduced to docility, and by moral suasion and self-sacrifice, rather than by force. With all the fervour of a new convert, John Conolly delivered a panegyric on the new asylum, the place where

> calmness will come; hope will revive; satisfaction will prevail. Some unmanageable tempers, some violent or sullen patients, there must always be; but much of the violence, much of the ill-humour, almost all the disposition to meditate mischievous or fatal revenge, or self-destruction will disappear. . . . Cleanliness and decency will be maintained or restored; and despair itself will sometimes be found to give place to cheerfulness or secure tranquility. [The asylum is the place] where humanity, if anywhere on earth, shall reign supreme.[123]

As this passage makes clear, a strong utopian strand runs through the early Victorian images of the asylum. The note of almost unbounded optimism, which its proponents so successfully communicated to their lay audience, was perhaps articulated in its purest form in Browne's vision of the asylum of the future:

> In place of multiplying individual examples of excellence, let me conclude by describing the aspect of an asylum as it ought to be. Conceive a spacious building resembling the palace of a peer, airy, and elevated, and elegant, surrounded by extensive and swelling grounds and gardens. The interior is fitted up with galleries, and workshops, and music rooms. The sun and the air are allowed to enter at every window, the view of the shrubberies and fields, and groups of labourers is unobstructed by shutters or bars; all is clean, quiet and attractive. The inmates all seem to be activated by the common impulse of enjoyment, all are busy, and delighted by being so. The house and all around appears to be a hive of industry. When you pass the lodge, it is as if you

122. *Ibid.*, pp. 199, 169.
123. John Conolly, *On the Construction and Government of Lunatic Asylums*, p. 143. On the convolutions of Conolly's thinking, and their connections to the vagaries of his career, see my *Social Order/Mental Disorder*, chapter 7.

had entered the precincts of some vast emporium of manufacture; labour is divided, so that it may be easy and well performed, and so apportioned, that it may suit the tastes and powers of each labourer. You meet the gardener, the common agriculturalist, the mower, the weeder, all intent on their several occupations, and loud in their merriment. The flowers are tended and trained, and watered by one, the humbler task of preparing the vegetables for table, is committed to another. Some of the inhabitants act as domestic servants, some as artisans, some rise to the rank of overseers. The bakehouse, the laundry, the kitchens, are all well supplied with indefatigable workers. In one part of the edifice are companies of straw plaiters, basket-makers, knitters, spinners, among the women; in another, weavers, tailors, saddlers, and shoemakers, among the men. For those who are ignorant of these gentle crafts, but strong and steady, there are loads to carry, water to draw, wood to cut, and for those who are both ignorant and weakly, there is oakum to tease and yarn to wind. The curious thing is, that all are anxious to be engaged, toil incessantly, and in general without any recompense other than being kept from disagreeable thought and the pains of illness. They literally work in order to please themselves, and having once experienced the possibility of doing this, and of earning peace, self-applause, and the approbation of all around, sound sleep, and it may be some small remuneration, a difficulty is found in restraining their eagerness, and moderating their exertions. There is in this community no compulsion, no chains, no whips, no corporal chastisement, simply because these are proved to be less effectual means of carrying any point than persuasion, emulation, and the desire of obtaining gratification. But there are gradations of employment. You may visit rooms where there are ladies reading, or at the harp, or piano, or flowering muslin, or engaged in some of those thousand ornamental productions in which female taste and ingenuity are displayed. You will encounter them going to church or to market, or returning from walking, riding, and driving in the country. You will see them ministering at the bedside some sick companions. Another wing contains those gentlemen who can engage in intellectual pursuits, or in the amusements and accomplishments of the station to which they belong. The billiard room will, in all probability, present an animated scene. Adjoining apartments are used as news-rooms, the politicians will be there. You will pass those who are fond of reading, drawing, music scattered throughout handsome suites of rooms, finished chastely, but beautifully and looking down upon such fair and fertile scenes as harmonize with the tranquillity which reigns within, and tend to conjure up images of beauty and serenity in the mind which are akin to happiness. But these persons have pursuits, their time is not wholly occupied in the agreeable trifling of conning a debate, or gaining so many points. One acts as an amanuensis, another is engaged in landscape

> painting, a third devolves to himself a course of historical reading, and submits to examination on the subject of his studies, a fourth seeks consolation from binding the books which he does not read. In short, all are so busy as to overlook, or all are so contented as to forget their misery.

'Such', Browne concluded triumphantly, 'is a faithful picture of what may be seen in many institutions, and of what might be seen in all, were asylums conducted as they ought to be'.[124]

Conjured up in these paragraphs is a quite extravagant portrait of ideal asylums as 'miniature worlds, whence all the disagreeable alloys of modern life are as much as possible excluded, and the more pleasing portions carefully cultivated'.[125] Here is a social universe that constitutes an organic, harmonious whole – a hierarchical order arrayed under its benevolent philosopher-king,[126] in which everyone knows and respects his or her place; where even the rage of madness is reigned in without whips, chains, or corporal chastisement, amidst the comforts of domesticity and the invisible yet infinitely potent fetters of the Tukean 'desire for esteem'; a community whose inhabitants 'literally work in order to please themselves', virtually without recompense, but so readily that 'a difficulty is found in restraining their eagerness . . .'; a Potemkin village notable for its absence of conflict and strife, wherein the conventional division of occupation and inclination along gender lines 'naturally' and reassuringly reproduces itself, while the lower orders of society coexist in harmony and tranquility with their betters.

Ideologically, of course, this was a vision of extraordinary resonance and attractiveness, particularly for a ruling class surrounded by alarming signs of increasing social friction and political discontent, and forced to confront the discord, disruptions, and divisiveness that were so central a feature of the Great Transformation. Confronted by the threats of Chartism and a militant working class; surrounded by the all-but-inescapable evidence of the devastating impact of industrial capitalism on the social and physical landscape; and themselves the authors of a New

124. W.A.F. Browne, *What Asylums Were, Are, and Ought to Be*, pp. 229–31.
125. Anon., 'Review of *What Asylums Were, Are, and Ought to Be*', *Phrenological Journal* 10, 53, 1836–7, p. 697.
126. Like his fellow alienists, Browne insisted throughout his career on the centrality and necessarily untrammeled power of the asylum superintendent. The delegation of autocratic power was a vital 'element in curative discipline. He is not merely a dispenser of advice and medicine, he is a moral governor, who identifies himself with the happiness as well as the health of those around; a referee in all disputes and difficulties; a depository of all secrets and sorrows; a source of pleasure, as well as of power and direction, and who gives a tone to every proceedings'. (Crichton Royal Asylum, *11th Annual Report*, 1850, p. 39.) A decade and a half later, the royal analogy is made overt: 'The power or government by which such communities are ruled should be monarchical. The details, as well as the principles, should emanate from one central will. . . .' (W.A.F. Browne, 'The Moral Treatment of the Insane', *Journal of Mental Science* 10, 1864, p. 334.)

Fig. 13 A drawing of the twelfth-night entertainment at the Hanwell Lunatic Asylum, taken from the *Illustrated London News*, 15 January 1848, demonstrating the happy effects of reform and the success of Conolly's non-restraint system. The entertainments at Hanwell were still segregated by sex – the female patients had their party on New Year's Eve. The cavernous corridor in which the inmates are seated at dinner was one of a number which ordinarily served as day rooms. The figures in the foreground include some of the asylum officers and Magistrates' Committee, with their wives.

Poor Law assailed by its critics (most memorably in Dickens's *Oliver Twist*) as the very embodiment of inhumanity and meanness of spirit, the Victorian governing classes were here offered the opportunity to redeem their society's progressive and humane character. Embracing the reformers' schemes and constructing the new realm of asylumdom would allow the replacement of violent repression, conflict, and strife by moral suasion, docility, and willing submission to authority, even among the depraved and unruly. The practical demonstration of the powers of 'reason and morality' when allied to a new kind of 'moral machinery' – these constituted a potent advertisement for the merits of reformed asylums run by practitioners initiated into the mysteries of moral treatment and medical psychology. And, like the Invisible Hand now regulating civil society, the new regime had the inestimable advantage of being a 'system . . . at once beautiful and self-operating. [The] presence [of keepers] is required to regulate the machine, but its motions are spontaneous . . .',[127] serving all

127. W.A.F. Browne, *What Asylums Were, Are, and Ought to Be*, p. 203.

but imperceptibly to secure 'the regulation and tranquillization of the unhealthy mind'.[128]

7. *The Reformers Triumphant*

Driven forward by such utopian reveries, and inspired with a limitless confidence in the merits of their chosen solution, the lunacy reformers now embarked upon the final and decisive phase of their campaign. Their aims remained those they had sought in 1815: the establishment on a compulsory basis of a network of public asylums for all pauper lunatics; and the creation of a full-time national inspectorate to supervise the whole system.[129] By the early 1840s, however, the chances of accomplishing these changes were greatly improved. The proponents of localism had fought the attempt to establish a similar measure of central control and direction over Poor Relief and lost. In the course of the 1830s, recalcitrant parishes had been brought around, frequently by the use of government patronage to bribe local elites. Having established the principle with reference to paupers as a whole, it was difficult to resist the conclusion that the treatment of a much smaller group like the insane ought to be subject to a similar authority, particularly since the parish was clearly too small a unit to deal with them on a specialized basis. Furthermore, the 1832 reform of Parliament had helped to weaken the power the entrenched local elites had hitherto enjoyed in the legislature, shifting the balance of political power towards the bourgeoisie – whose interests were best served by uniform national policies.

Meanwhile, as we have seen, the literate public had been provided with a surfeit of tracts arguing the superior merits of the asylum as a solution to the problems of insanity. The Metropolitan Commissioners had made use of their periodic reports to stress the improvements which could be brought about by assiduous and frequent inspection; and to contrast the marked amelioration of conditions in the metropolis with the stagnation or retrogression in the provinces, where 'the salutary provisions of the Act by which it is required that all houses . . . shall be annually licensed and periodically visited and reported by the Visiting Magistrates, have been in

128. *Ibid.*, p. 213.

129. These aims bore the clear stamp of Benthamite influence. The scheme for expert, full-time inspectors employed by the central government to provide a check on local administrators paralleled the system elaborated for the reform of the Poor Law by Chadwick and Nassau Senior in the 1832 Poor Law Inquiry, and the Poor Law Amendment Act of 1834. The insistence on the value of the institution had received the approbation of the master himself (Jeremy Bentham, *Panopticon, or the Inspection House*, London: Payne, 1791), while the Poor Law Commission had endorsed existing asylums as a model for the 'separate and appropriate management of the various sub-classes of the incompetent poor.' Similarly, the idea that the defects of existing asylums were due to the lack of efficient central oversight had its parallel in the Poor Law reformers' rationalization of the horrors of the pre-1834 general mixed workhouse as due to maladministration and the lack of central direction.

great degree neglected or violated . . .'.[130] Finally, the discovery at the Lincoln Asylum that the insane could be managed without resort to any form of mechanical restraint whatsoever, and the successful implementation of such a policy at Hanwell, the largest of all public asylums, had been promoted as a dramatic demonstration of the reformers' contention that the establishment of public asylums would eliminate the horrors of the old madhouse regime.[131] When the *Westminster Review* sought to advance the cause of reform, it simply summarized the content of a number of asylum reports, and concluded triumphantly that evidences of success 'crowd the pages of the Reports before us, and they contain not the prospective theories of an imaginary philanthropy, but the practical results of a tested system'.[132]

On other grounds, too, the desirability, even the necessity of reform had been urged upon the governing classes. Running through the burgeoning specialist literature of the early nineteenth century were pronouncements on three crucial issues: how much insanity existed in the community; what caused people to go mad; and the differential social incidence of mental alienation. Naturally enough, the more extensive the legions of the mad, the more pressing the question of lunacy reform became. The more closely insanity was bound up with the conditions of modern civilized existence, the more threatening the future prospects for society at large, and the more vital the necessity for expert and effective intervention to mitigate the problem. And the more susceptible the rich and powerful to the ravages of mental disorder, the more urgently their own self-interest commanded them to adopt the reformers' prescriptions, lest they wake some morning to find themselves incarcerated in one of those 'wild and secluded abodes of human misery' to which the mad were traditionally consigned.

The experts' conclusions on all these points were scarcely reassuring. Conceding the defects of existing statistics, their consensus was nonetheless 'that a much greater number of cases is known to exist, and to require treatment, than formerly', and that the numbers of the insane appeared to be increasing at a far more rapid rate than the population in general.[133] Nor should one be surprised to find 'the poisoned stream larger, and wider, and deeper', given 'the too palpable multiplication of the causes

130. *Annual Report of the Metropolitan Commissioners in Lunacy*, 1838, p. 6.
131. For discussions of the 'non-restraint' movement, see my *Social Order/Mental Disorder*, pp. 188–97; and Nancy Tomes, 'The Great Restraint Controversy: A Comparative Perspective on Anglo–American Psychiatry in the Nineteenth Century', pp. 190–225 in W.F. Bynum, R. Porter, and M. Shepherd (eds), *The Anatomy of Madness*, Vol. 3, London: Routledge, 1988.
132. *Westminster Review* 37, 1842, p. 310.
133. W.A.F. Browne, *What Asylums Were, Are, and Ought to Be*, pp. 54–5; see also Andrew Halliday, *A General View of the Present State of Lunatic Asylums in Great Britain and Ireland*, London: Underwood, 1828; Jean Etienne Dominique Esquirol, *Mental Maladies*, Philadelphia: Blanchard and Lee, 1845. For a more sceptical view, see G.M. Burrows, *Commentaries*.

which produce mania'.[134] Wherever 'the sources of moral agitation and excitement are most abundant, there', it was confidently asserted, 'will the proportion of insanity be the highest'[135] – grim news indeed for a social order characterized by wrenching social change and upheaval, and for the commercial, manufacturing, and professional elites who so relentlessly sought to exploit the resulting opportunities for social and material advancement.

The Rousseauist myth that the noble savage was somehow immune from the ravages of insanity was widely canvassed by Regency and early Victorian writers on the subject.[136] Thomas Beddoes spoke of nations 'civilized enough to be capable of insanity'[137] and Alexander Morison reported that madness appeared 'to be very small in South America, and among the Indian tribes, &c. and to be very considerable in China. It is . . . probable', he gravely concluded, 'that the increasing civilization and luxury of this country, co-operating with hereditary disposition, tends rather to increase the numbers in proportion to the population'.[138] Others were less restrained in their language: the London physician, John Reid, announced in apocalyptic tones, that 'Madness, strides like a Colossus over this island . . .'[139]; and W.A.F. Browne echoed his warning: 'as we recede, step by step, from the simple, that is, the savage, manners of our ancestors, and advance in industry and knowledge and happiness, this malignant persecutor strides onward, signalizing every era in the social progress by an increase, a new hecatomb of victims'.[140]

Here was a paradox, particularly for an audience which, as a matter of course, equated a 'higher' civilization with progress in all spheres of existence. Moreover, if the connection were to prove an inseparable and inevitable one, it was a peculiarly discomforting paradox, for it implied that one could look forward to a rapid and persistent rise in the numbers of mad folk. Fortunately, some reassurance was forthcoming. The increase occurred, not inevitably, but

134. W.A.F. Browne, *What Asylums Were, Are, and Ought to Be*, pp. 54–5.
135. *Ibid.*, p. 63.
136. Of course, the connections between artificiality, over-stimulation, luxury, and madness had been a staple of speculation on the causes of nervous distempers at least since the publication of George Cheyne's *The English Malady*, London: Wisk, Ewing, and Smith, 1733. For a general discussion of the persistence of this belief as a staple of nineteenth-century psychiatric theorizing, see D.J. Mellett, *The Prerogative of Asylumdom*, New York: Garland, 1982, pp. 64–9.
137. Thomas Beddoes, *Hygeia*, Bristol: Mills, 1802, Vol. 2, p. 40.
138. A. Morison, *Outlines of Lectures on Insanity*, Edinburgh: Lizars, 1825, p. 73.
139. John Reid, 'Report of Diseases', *The Monthly Magazine* 25, 1808, p. 166.
140. W.A.F. Browne, *What Asylums Were, Are, and Ought to Be*, p. 52. The conclusion was echoed by an anonymous reviewer: 'In that state of society which is now termed civilized, the sources of mental excitation and disease are almost beyond computation.' Anon., 'Review of *What Asylums Were, Are, and Ought to Be*', *Phrenological Journal* 10, #53, 1836–7, p. 691.

> because the enjoyments and blessings of augmented power are abused; because the mind is roused to exertion without being disciplined, it is stimulated without being strengthened; because our selfish propensities are cultivated while our moral nature is left barren, our pleasures becoming poisonous; and because in the midst of a blaze of scientific light, and in the presence of a thousand temptations to multiply our immediate by a sacrifice of our ultimate gratifications, we remain in the darkest ignorance of our own mind. . . .[141]

If society could but be brought to listen to those who were lifting the veil of ignorance surrounding the subject (and who were simultaneously developing expertise in restoring the mad to sanity) the worst might be averted. Inaction, however, might well prove fatal.

Nor could wealth or social position provide any security against the inroads of insanity. Spurzheim's even-handed claim that 'no one is secure from [going mad]; . . . rich and poor, the laborious and sober labourer and his master who indulges in scenes of luxury are all equally liable to this affliction'[142] was soon supplanted by far more ominous warnings from no less an authority than Esquirol that madness was more usually a disease of the rich. Even the growth in the trade in lunacy itself was cited by some as evidence of the disproportionate susceptibility of the well-to-do.[143] Whilst the agricultural population, and particularly the rural poor, 'is to a great degree exempt from insanity', the bourgeoisie and the plutocrats enjoyed no such immunity from its depredations, exposed as they were 'to excitement . . . and . . . to the formation of habits of thought and action inimical to the preservation of serenity and health'.[144] The upwardly mobile were perhaps the most vulnerable of all: 'Pianos, parasols, *Edinburgh Reviews*, and Paris-going desires, are now found among a class of persons who formerly thought these things belonged to a different race; these are the true source of nervousness and mental ailments.'[145]

If the most successful in the race for consumption goods and competitive advantage were simultaneously now pictured as the most susceptible to the ravages of madness, a similar vulnerability appeared to characterize the dominant sex: not for the first time, indeed, for as Michael MacDonald has shown, in sixteenth- and seventeenth-century England, 'Madness [also] wore a masculine visage. . . . [Under the Tudors and Stuarts,] society was more concerned about men who fell insane than women . . . [and t]he stereotypical lunatic was Mad Tom, a tattered beggar, hardly

141. W.A.F. Browne, *What Asylums Were, Are, and Ought to Be*, pp. 52–3.
142. Johann Gaspar Spurzheim, *Observations on the Deranged Manifestations of the Mind, or Insanity*, London: Baldwin, Craddock, and Joy, 1817.
143. W.A.F. Browne, *What Asylums Were, Are, and Ought to Be*, p. 61.
144. *Ibid.*, pp. 56–7.
145. David Uwins, *A Treatise on Those Disorders of the Brain and Nervous System, Which Are Usually Considered and Called Mental*, London: Renshaw and Rush, 1833, p. 51.

better than a beast.'[146] But such presumptions had been overturned gradually over the course of the eighteenth century, and by the dawn of the Victorian era, alienists generally portrayed women as biologically weaker and more liable to go mad;[147] and the broader cultural images of insanity had been securely tied to the female of the species: in novels, in drama, in poetry, in painting, in popular ballads, in opera, it was disproportionately women who stood as the emblems and exemplars of irrationality.[148] And yet the newly collected statistics of insanity suggested a quite different picture. John Thurnam's findings, for instance, indicated that men outnumbered women in English asylums by about thirty per cent, and other data suggested that the discrepancy between the two had widened during the preceding quarter century, to the disadvantage of men.[149]

146. Michael MacDonald, 'Women and Madness in Tudor and Stuart England', *Social Research* 53, 2, 1986, p. 262. The predominance of men among the ranks of the mad was as apparent in practice as at the level of cultural stereotypes, notwithstanding the conventional assumption that women's 'reason was feebler than men's and less able to constrain unruly passions . . . descriptions of madness and frenzy in sixteenth- and seventeenth-century medical books always refer to the typical patient as male'. *Ibid.*, pp. 263–4, 267–8.
147. See, for instance, the discussions in John Haslam, *Observations on Insanity . . .*, p. 108; in idem, *Observations On Madness and Melancholy*, pp. 245–9; and in J.G. Spurzheim, *Observations on the Deranged Manifestations of the Mind, or Insanity*, pp. 162–3. The special causes of insanity in women were alleged to include disorders associated with menstruation, parturition, and 'preparing nutriment for the infant'. Women, too, were more exposed to disappointments in life, and were more often the victims of circumstances, while having fewer resources with which to cope with life's setbacks. Finally, in contrast to men, their feelings and emotions were generally stronger than their intellects, heightening their susceptibility to madness.
148. See, for example, the numerous portraits of 'Crazy Jane' or 'Crazy Kate'; Bertha Mason, the quintessentially gothic figure of 'the madwoman in the attic'; Sir Walter Scott's Lucy Ashton, subsequently the heroine of eight different operas, most notably Donizetti's *Lucia di Lammermoor*; and the ascension of Shakespeare's Ophelia to almost the status of a cult figure among early nineteenth-century romantics. Elaine Showalter has suggested that this association of irrationality with the female marked a cultural shift occurring over the course of the second half of the eighteenth century – that in the Augustan age by contrast, 'the cultural image of the lunatic was male'. Elaine Showalter, *The Female Malady: Women, Madness and English Culture, 1830–1980*, New York: Pantheon, 1985, p. 8.
149. *Cf.* John Thurnam, *Observations and Essays on the Statistics of Insanity*, London: Simpkin Marshall, 1845, pp. 145–55. *Cf.* the earlier findings of Richard Powell, 'Observations Upon the Comparative Prevalence of Insanity at Different Periods', *Medical Transactions* 4, 1813, pp. 131–59, that admissions to madhouses in the metropolis between 1804 and 1808 comprised 1,128 men and 1,000 women. Similarly, a Parliamentary Return for 1826 recorded 4,461 males and 3,443 females admitted to private madhouses between 1815 and 1824. Not everyone took these statistics at face value. Thomas Bakewell, who reported that the number of female patients sent to his private madhouse at Spring Vale was 'only half the number of males', was nonetheless sceptical of the claim that this corresponded to the actual incidence of the disorder. Because of the cost of confinement, 'none can be expected but the most confirmed, aggravated cases' and given that 'females may be better managed at home, therefore the expense is not incurred'. *A Letter to the Chairman of the Select Committee . . . Appointed to Inquire into the State of Madhouses*, p. 61.

To the extent that these assertions about the social geography of madness secured a measure of public credibility and acceptance, they provided the men who formed the governing classes with still more powerful motives to embrace the cause of lunacy reform. To be sure, once a network of reformed asylums had been constructed, the overwhelming bulk of the patient population would turn out to be drawn from the ranks of the poor and the disenfranchised, and past mid-century, female pauper lunatics soon began to outnumber their male counterparts.[150] But in the early 1840s, the best professional opinion suggested that it was the educated, the wealthy, the most cultured segments of the community (and quite possibly the men, who after all were most directly exposed to the noxious effects of heightened competition, speculation and ambition) who had the most to fear from the spectre of madness – all of which surely heightened the receptivity of an elite who shared these characteristics to the claims and clamour of the lunacy reformers.

With the ground for advancing the reformers' plans so well prepared in advance, the Metropolitan Commissioners now went beyond mere complaints of the ineffectiveness of provincial inspection. On 17 March

150. The 1871 census, for instance, showed that while the ratio of women to men in the population at large was 1,056:1,000, in asylums it was 1,182:1,000. Among the upper classes, males would continue to outnumber female patients almost till the close of the century, but there were already 1,242 female pauper lunatics for every 1,000 male lunatics. Elaine Showalter, who cites these figures (see *The Female Malady*, pp. 52, 259), deploys them as a central part of her argument that during the nineteenth century, madness became 'the female malady'. I have suggested elsewhere (*Social Order/Mental Disorder*, chapter 11) that this places more weight on these data than is really warranted. Victorian alienists themselves remarked upon the phenomenon, but suggested (correctly in my view) that a substantial fraction of the difference disappeared once one took account of women's greater longevity, and their longer average stay in the asylum. (Men were discharged more rapidly on the average, perhaps because families more desperately needed them as breadwinners.) For discussion, see, for example, John Conolly, *On the Construction and Government of Lunatic Asylums*, pp. 146–50; and Edgar Sheppard, *Lectures on Madness in Its Medical, Legal, and Social Aspects*, London: Churchill, pp. 3–5. What is intriguing and lends weight to the feminist critique of psychiatry is surely something rather different: the fact that, notwithstanding the nearly equal propensity of the two sexes to go mad, Victorian alienists developed theories of a differential, gender-based etiology for mental disturbance. From neurological portraits of females as possessed of nervous systems of greater refinement and delicacy (and hence more susceptible to breakdown) to gynaecological theorizing about peculiarly intimate ties between a woman's brain and her reproductive organs, medical men constructed images of creatures whose physiological equipment was of surpassing fragility, liable to give way at any moment under the strains of modern life or the unavoidably perilous passage through puberty, pregnancy, parturition, lactation, menstruation, and the menopause. In the process, the constriction of women's lives, their legal powerlessness, and their economic marginality – the central features of existing social relations between the sexes, thus received the sanction of science. For a particularly egregious but by no means unrepresentative example of the genre, see Henry Maudsley, 'Sex in Mind and Education', *The Fortnightly Review* 21, 1874, pp. 466–83. For the ominous consequences for treatment which could flow from such assumptions, see Andrew Scull and Diane Favreau, '"A Chance to Cut is a Chance to Cure": Sexual Surgery for Psychosis in Three Nineteenth Century Societies', in S. Spitzer and A. Scull (eds), *Research in Law, Deviance, and Social Control*, Vol. 8, Greenwich, Connecticut: JAI Press, 1986, pp. 3–39.

1842, Lord Granville Somerset, the chairman of the Commission, introduced a bill into the Commons to extend its powers for three years to allow it to carry out a comprehensive inspection of all asylums and madhouses in the country.[151] The inspections which followed the bill's passage in early August were extremely extensive and detailed: asylum records were checked for accuracy and conformity to the legal requirements of the various lunacy statutes; close attention was paid to the condition of the building in which the patients were housed; questions were asked about the origins of each asylum and its current sources of financial support; inquiries were made into the role of the medical attendants, and the general administration of the asylum; patients were questioned about the treatment they received; and so on. Two years later, the findings were embodied in a comprehensive report, detailing the condition of every asylum in England, with sections on the nature of insanity and its proper classification, consideration of the arguments for and against the non-restraint system, and a discussion of the admission of pauper lunatics from workhouses, concluding with detailed recommendations about the direction reform should take.

The Commission found that 'the asylums thus brought before our view exhibit instances of about every degree of merit and defect'. Institutions in several categories were found to be reasonably satisfactory, but in a number of instances there were 'Asylums and Licensed Houses which deserve almost unqualified censure.'[152] It was these extreme cases which provided the best argument for change, and which were widely cited in summaries of the Report written for popular consumption, as well as in the parliamentary debates on the legislation which the reformers introduced. The types of abused uncovered differed little from those found by parliamentary inquiries earlier in the century: At West Auckland,

> each sex had only one sitting room, with windows that did not admit of any prospect from them, and the violent and the quiet, and the dirty and the clean were shut up together. There was only one small walled yard, and when one sex was in it, the other was locked up. . . . In the small, cheerless day-room of the males, with only one (unglazed) window, five men were restrained by leg-locks . . . and two more were wearing, in addition, iron hand-cuffs and fetters from the wrist to the ankle: they were all tranquil. The reason assigned for this coercion was, that without it they would escape. . . . Chains were fastened to the floor in many places, and to many of the bedsteads. The males slept two to a bed . . .[153]

151. The legislation also added two more barristers and two more doctors to the Commission, strengthening the 'professional' element of the inspectorate.
152. *Report of the Metropolitan Commissioners in Lunacy to the Lord Chancellor*, London: Bradbury and Evans, 1844, pp. 7, 46.
153. *Ibid.*, pp. 53–4.

– something which shocked Victorian sensibilities. At a house in Derby,

> the straw in the paupers' beds was found filthy, and some of the bedding was in a disgusting condition from running sores, and was of the worst materials, and insufficient. Two cells, in which three sick epileptic paupers slept, were damp, unhealthy, and unfit for habitation. The beds of some of the private patients were in an equally bad state.[154]

Another house was described as

> deficient in every comfort and almost every convenience. The refractory patients were confined to strong chairs, their arms being also fastened to the chair. One of these – a woman – was entirely naked on both the days the Commissioners visited the asylum, and without doubt during the night. The stench was so offensive that it was almost impossible to remain there.[155]

A major theme of the Report was that only the creation of a powerful national inspectorate on a permanent basis would ensure the elimination of such abuses. Local inspection by magistrates was simply inadequate. All the asylums the Commissioners termed 'utterly unfit' were private asylums in the provinces receiving paupers, licensed by the local magistrates and hitherto only visited by them.[156] No house in the London area, where the Commissioners themselves had had complete control for a decade and a half, exhibited comparable defects. Warburton's, for instance, which in 1828 was as bad as any of the provincial houses, was now apparently transformed:

> We have visited few, if any, receptacles for the insane in which the patients were more kindly or more judiciously treated . . . the abuses which existed previously to the year 1828 led to the introduction of a system of visitation by commissioners in the metropolitan district. The houses at Bethnal Green, which were among the worst, now rank with the best receptacles for the insane.[157]

Only continued vigilant inspection could ensure such results: 'we . . . are convinced that some of these very houses of which we now speak in terms of commendation, would soon become scenes of great abuses were it not for the checks interposed by constant and watchful visitation to which they are subjected'.[158]

154. *Ibid.*, p. 56.
155. *Ibid.*
156. These included: West Auckland and Wrekenton in County Durham; Green Hill House in Derbyshire; Lainston House, Winchester and Grove Place, Nursling, Hampshire; Kingsdown House, Wiltshire; Plympton House, Devon; Moor Cottage, Yorkshire; and West Malling Place, Kent.
157. *Report of the Metropolitan Commissioners in Lunacy*, 1844, p. 44.
158. *Ibid.*, p. 35.

In emphasizing the gains inspection could produce, as they had to in order to convince others of the necessity for a national inspectorate, the Metropolitan Commissioners threatened to undermine their own case for the other major reform they sought to introduce. For if effective supervision alone could raise an institution from the ranks of the worst to among the best, why was it not sufficient by itself? Why was it necessary to introduce a measure to compel counties to make a large capital investment in the erection of sufficient public asylums to accommodate every pauper patient? After all, their own description of existing county asylums had been a lukewarm endorsement at best:

> It is apparent . . . that although a few . . . are well adapted to their purpose, and a very large proportion of them are extremely well conducted; yet some are quite unfit for the reception of the insane, some are placed in ineligible sites, some are deficient in the necessary means of providing outdoor employment for paupers, some are ill-contrived and defective in their internal accommodations, some are cheerless and confined in their yards and airing grounds, and some are larger than seems consistent with the good management of their establishments and the proper health and care of their inmates.

But county asylums offered a decisive advantage over the private system – the judgement and conduct of those running them could not be perverted by considerations of personal profit. The defects of existing institutions of this sort were largely the product of inexperience and could readily be eliminated by more expert guidance on the part of the central authorities. Consequently, after Ashley had summarized the worst abuses the Report had uncovered for the House of Commons, he assured his listeners that 'to correct these evils there was no remedy but the multiplication of county asylums'. Experience showed that many counties were deterred from performing their manifest duty by considerations of expense, so providing adequate accommodation for pauper lunatics at public expense ought henceforth to be made compulsory.

Throughout the nineteenth century, it was an article of faith among those who dealt with lunatics that the deranged were more readily restored in the early stages of the disorder, so that delay in help could prove disastrous. Whatever its scientific merits, as ideology such a belief was of great assistance to the asylum doctors. It helped, of course, to explain the low proportion of their patients they managed to restore to sanity. More important than that, it did so in a way which demonstrated that the failure lay, not with the asylum, but with the public. If asylums did not cure, it was because the public did not send lunatics to them fast enough; since existing asylums were already flooded with chronic cases, the situation could only be remedied by building still more asylums; and given that the supply of asylum beds could never catch up with the demand, which

appeared to expand in proportion with the number of available beds, the proposition would never be tested.

Here was the Commission's final and decisive argument for more public asylums:

> At the Retreat, York, at the Asylums of Lincoln and Northampton, and at the Asylum for the County of Suffolk, tables are published, exhibiting the large proportion of cures effected in cases where patients are admitted within three months of their attacks, the less proportion when admitted after three months, and the almost hopelessness of cure when persons are permitted to remain in Workhouses or elsewhere, and are not sent into proper asylums until after a lapse of a year from the period when they have been first subject to insanity.[159]

At the same time, the testimony from county asylum superintendents exhibited their virtually 'unanimous opinion that pauper lunatics are sent there at so late a period of their disease as to impede or prevent their ultimate recovery'.[160] The problem was that 'even if there did exist on the part of the guardians and overseers of the poor a full knowledge of the importance of early treatment, and the most earnest desire to avail themselves of its advantages, throughout almost the whole of England, and in the whole of Wales, there is so great a want of accommodation for the reception of the insane that they could not carry their views into effect'.[161]

The Report was completed just before the close of the 1844 session of Parliament. On 23 July, Ashley, in a long speech, outlined its major conclusions and urged legislative enactment of its central recommendations. Though action was postponed until the following year, this speech did help to draw attention to the Report's findings. Later the same year, the Tory *Quarterly Review* published a long article on the subject, endorsing all the Commission's suggestions and calling for their swift passage into law.[162] Early in 1845, its Benthamite counterpart, the *Westminster Review*, followed suit.[163] Both pieces dwelt at length on the intolerable conditions which had been found, and concluded that even on strictly economic grounds, the Commission's proposals deserved support. For 'the very great probability of cure in the early stages of insanity', provided lunatics received prompt asylum treatment, suggested that making proper provision for the insane population of the country would ultimately reduce the numbers needing support at public expense. By the time Ashley introduced legislation to give effect to the Report's recommendations, in June 1845, informed opinion had moved decisively in his favour. The two bills he proposed both received government backing and passed swiftly

159. *Ibid.*, pp. 80–1.
160. *Ibid.*, p. 80.
161. *Ibid.*, p. 82.
162. 'Report on the Treatment of Lunatics', *Quarterly Review* 74, 1844, pp. 416–47.
163. 'Lunatic Asylums', *Westminster Review* 43, 1845, pp. 162–92.

through both Houses of Parliament, becoming law on 4 and 8 August 1845.

The first, the Lunatics Act of 1845 (8 & 9 Victoria *c.* 100), established a permanent national Lunacy Commission, with power to make detailed and frequent inspections of all types of asylums – whether public, private, or charity foundations. The second Act (8 & 9 Victoria *c.* 126) made the erection of county and borough asylums to house pauper lunatics compulsory. In keeping with the recommendations of the Metropolitan Commissioners, and to ensure that the presence of large numbers of chronic patients would not interfere with the asylum doctors' ability to produce the cures they had promised, counties were authorized, though not instructed, to erect separate, less costly buildings for chronic lunatics. And to ensure that accommodation was not overcrowded or insufficient, counties which failed of their own volition to make adequate provision for their insane population could be compelled to do so.

8. *The Ideal and the Reality*

By 1845, therefore, the reformers had been successful in securing the enactment of both the key elements in their plan to remedy the condition and treatment of the insane. The insane had been sharply distinguished from other types of indigent and troublesome people and the asylum had been officially recognized as the most suitable place for them. A whole network of such institutions was now to be created at public expense, within whose walls all cases of lunacy were to be treated by members of the medical profession who had made or would make the treatment of mental disturbance their specialty. Most cases would now be cured. It was, the reformers agreed, a triumph for science and humanity.

Experience, however, was already suggesting that this optimism about the future was misplaced. As early as 1845, under the multiple and reinforcing pressures which derived from the difficulties associated with the routinization of reform, the economies imposed by cost-conscious local authorities, and the impact of an overwhelmingly lower-class clientele, there were clear signs of the collapse of the very things the reformers thought were indispensable to the success of the whole enterprise. The asylum doctors themselves contributed to this process. Bowing to political and social realities, the medical superintendents of county asylums began to compromise and water down their requirements, always consoling themselves with the thought that 'the worst asylum that can at this day by possibility be conceived, will still afford great protection' to the poor lunatic, compared to the treatment he would get elsewhere.[164]

County asylums had proved expensive to build and expensive to operate. In 1843, the average weekly cost of caring for a pauper lunatic in

164. J. Thurnam, *Observations and Essays on the Statistics of Insanity*, p. 104.

a public asylum was 7s. $6\frac{3}{4}$d., a sum which did not include any provision for the capital cost of the building itself. By contrast, lunatics were maintained in the community for only 2s. $7\frac{1}{4}$d., roughly what it cost to keep a pauper in an ordinary workhouse. Hostile Poor Law officials alleged that 'a vast sum of money has been thrown away in the erection of [asylums],' and that running them more nearly along the lines of workhouses would enable lunatics to 'be kept at one half or a third or a fourth of the expense at which they are now kept'.[165]

By way of justification of their 'excessive' expenditures on paupers, the asylum's proponents produced a number of counter-arguments. To begin with, the inmates they had to deal with were far more difficult and intractable as a class than the average workhouse inmate, and required closer and more continuous supervision, all of which raised costs. Then again, the workhouse was specifically designed to deter the able-bodied, undeserving poor from seeking public relief, so that within its walls the strictest regard for economy was highly functional. Not only was it unfair to subject those who were not responsible for their own condition to such treatment, it was also likely to be counterproductive, making permanent what skilful attention might cure. The false economy of the Poor Law officials overlooked the fact that it was cheaper in the long run to pay a higher sum for a few months, and have the patients restored to sanity and productivity, than to provide 'inexpensive' custodial care for a lifetime.

Many magistrates and parish officials remained stubbornly unconvinced, with the result that, before the 1845 Act made their erection compulsory, the majority of counties failed to build their own asylums. The magistrates were reluctant to impose extra local taxes which would fall heavily on their own class, and were strengthened in their resolve by the parish Guardians of the poor. Where asylums *were* erected, these men sought to keep costs to a minimum and to pare down the 'frills and luxuries' the reformers wanted, arguing that paupers could not appreciate them anyway. Even those who agreed that 'nothing [should] be neglected which may tend to facilitate the recovery of the patients' were inclined to view the more costly aspects of moral treatment with a somewhat jaundiced eye. They insisted that 'when this accommodation is to be produced for the poor and paid for out of Poor Rates, it is imperative that it should be done with the strictest eye to economy'.[166]

Realizing that 'the main impediment in the way of constructing county asylums has been . . . and at present is, the fear of the enormous expense supposed to be necessarily attendant upon such undertakings',[167] the reformers now discovered that some of the things they had formerly

165. *Select Committee on the Poor Law Amendment Act*, 1838, quoted in K. Jones, *Lunacy, Law, and Conscience, 1744–1845*, pp. 165–6.
166. *Memorial from the Parish of Brighthelmston, Sussex, Concerning the Proposed County Asylum*, 1845, in manuscript in the East Sussex Record Office, Lewes, QAL 2/3/6.
167. Ashley, in *Hansard* 81, Third Series, 1845, column 190.

insisted on could be safely discarded. It was conceded that 'the first object that should be kept in view, after providing for the comfort and health of the patients, is economy; for, after all that can be said of the feelings of humanity towards this unfortunate class of our fellow-creatures, their sufferings are too much out of sight to create that sympathy for them'.[168] William Ellis, now superintendent at Hanwell, showed a keen awareness of the consequences of this concession for those who, like him, sought to extend the benefits of asylum treatment to all of the insane. As he revealingly put it, 'It becomes necessary to show that to render them efficient need cost very little more than to neglect them'[169] – a necessity which did not disappear because it unavoidably meant that the fundamental principles of moral treatment would have to be compromised in the process.

The Metropolitan Commissioners had found grave defects in the siting, construction, architecture, and facilities of existing county asylums. Apparently all these deficiencies could be overcome while making radical economies in the construction of future asylums, for Ashley now asserted that while the buildings at Hanwell and the Surrey Asylum had cost 160 pounds and 245 pounds a head respectively, perfectly adequate accommodation for curable cases could be built for 80 pounds. For chronic cases, the cost could be even less. The 1844 Report provides us with some notion of how this was to be achieved: 'Although we have no wish to advocate the erection of unsightly buildings, we think that no unnecessary cost should be incurred for architectural decoration; especially as these Asylums are erected for those who, when in health, are accustomed to dwell in cottages.' Ellis gives an indication of what dispensing with surplus architectural decoration would mean in practice: 'In asylums designed for paupers only, it is unnecessary to have any plaster on the walls; limewash on the bricks is all that is required.'[170] The cheerful and pleasing architecture, which in initial formulations of moral treatment played such an important role in creating and sustaining the optimistic and familial atmosphere so essential to success, was now, with the blessing of the asylum doctors, transformed into an 'unnecessary cost'. Asylum buildings became increasingly monotonous, drearily functional, prison-like.

If the reformers had collaborated, with little apparent discomfort, in this departure from their earlier ideals, they were somewhat less willing to accept some of the other changes the magistrates insisted on. One of the most important of these, symbolically and actually, was the question of asylum size. Tuke and his followers had made a powerful case for the contention that small asylums were a necessary, even if not a sufficient, condition for individualized care of patients. Several of the earliest county

168. William Ellis, *A Treatise on . . . Insanity*, p. 267.
169. *Ibid.*
170. *Ibid.*, p. 275.

asylums had conformed quite closely to these designs and to the model provided by the York Retreat. Lincoln was built for sixty patients, Nottingham for eighty, Gloucester for 120. Even those erected in highly urbanized counties were initially quite modest in size – the West Riding Asylum at Wakefield had space for 150, and that at Lancaster for 170.

The small intimate institution did not survive for long. The influx of a horde of derelict paupers brought the demise of the notion that the asylum should be a substitute household. Instead, local magistrates insisted on taking advantage of presumed economies of scale. Until well into the twentieth century, the average size of county asylums grew almost yearly. While the trend towards larger asylums became well-nigh universal after 1845, it was already well marked by then. Five of the fifteen county asylums built by 1844 contained well over the recommended maximum of 200 patients: the Kent Asylum contained 300; Surrey 360; Wakefield 420; Lancaster 611; and Hanwell as many as 975, with a nominal capacity of 1,000. The profession relaxed its standards somewhat, but not sufficiently. When the Metropolitan Commissioners censured the Middlesex magistrates, alleging that their policies were directly responsible for the 'evils and inconveniences which have been experienced at Hanwell owing to its extreme size', they were merely providing a faithful reflection of the best professional opinion. They had no effect. As Conolly put it, 'the magistrates go on adding wing to wing and story to story, contrary to the opinion of the profession and to common sense, rendering the institution most unfavourable to the treatment of patients, and their management most harassing and unsatisfactory to the medical superintendent'.[171] All the powers of persuasion the asylum doctors could muster, all their elaborate statistical proofs that it was cheaper to cure in a small asylum than to immure in a vast custodial warehouse, were in vain. The magistrates who employed them remained unconvinced, and given the choice between hypothetical cures and concrete savings, they consistently chose the latter.

'[Hanwell] was designed for three hundred patients; but, with greatly economizing the room, and making use of a part of the basement, it has been fitted to accommodate six hundred and fifteen.'[172] Whatever the consequences of this for the patients' daily existence, the magistrates were clearly delighted, for during this same period, the weekly cost of each patient's maintenance fell from 9s. a week to only 5s. 10d.[173] In similar fashion, the buildings at the Wakefield Asylum, originally intended for 150 patients, were made to accommodate 296.[174] It was not until well into

171. Letter from John Conolly to Sir James Clark, quoted in the *Edinburgh Review* 131, 1871, p. 221.
172. Ellis, *A Treatise on . . . Insanity*, p. 283.
173. Metropolitan Commissioners in Lunacy, *1844 Report*, pp. 86–7.
174. Samuel Tuke, 'Introductory Observations', in M. Jacobi, *On the Construction and Management of Hospitals for the Insane*, p. xxiv.

the second half of the nineteenth century that these 'monstrous asylums' became general, but the larger asylums of the 1830s and 1840s already exhibited many of the features which would later be typical of the asylum system as a whole.[175] Large numbers from the outset required the development of an orderly bureaucratic routine. Prior to the opening of Hanwell, for example, the magistrates drew up a detailed and elaborate code of rules. The duties and qualifications of each of the officers, keepers, and servants were carefully specified, and lines of authority drawn. Efforts were made to provide for every contingency: there were rules covering how dead patients were to be buried; regulations about when patients were to get up and go to bed, and when they were to eat ('Patients will breakfast at eight, dine at one, and sup at seven'); the diet tables were so detailed, they even included instructions as to how the ingredients used in the gruel given out at breakfast and supper were to be prepared.

This system of fixed rules was enforced by a discipline based on solitary confinement and deprivation of privileges, rather than whips and chains. At first, these were supplemented by the use of the shower bath as a punishment, by mild mechanical restraint, or by 'the terror . . . of the electrifying machine'. As superintendents became accustomed to managing such large numbers, however, they began to realize that they could do without mechanical restraint. They discovered in a pragmatic way what Goffman has so elegantly shown the rest of us, that in the context of a life in a total institution, manipulation of small rewards and privileges, seclusion, or the threat of removal to a 'worse' ward have such profound implications for the self that more overtly punitive strategies are seldom required.

The degree of regimentation needed to administer an institution of 600 or 1,000 inmates ensured that such asylums would be the virtual antithesis of their supposed inspiration, the York Retreat. To Tuke, moral treatment had meant the creation of a stimulating environment where routine could be sacrificed to the needs of the individual. Here the same term disguised a monotonous reality in which the needs of the patients were necessarily subordinated to those of the institution; indeed, where a patient's needs were unlikely even to find expression.

9. *Controlling the Uncontrollable*

All of this suggests that the county asylums were an aberrant development, a deviation from the ideals of moral treatment, as in a sense, they were. But in an ironic way they also embodied some of the core principles of the new approach. They showed, more clearly than did the Retreat, the true character of the 'reform' Tuke had unwittingly fostered.

175. The increasing size of asylum populations and the reasons for the apparent rise in the number of mad people in the course of the nineteenth century are examined at some length in chapter 7.

The central fact about moral treatment, after all, and the reason for its immediate appeal, was the way it demonstrated that the most repellent features of existing madhouses were actually unnecessary cruelties. One of Tuke's proudest claims was that 'neither chains nor corporal punishment are tolerated, on any pretext in this establishment'.[176] Instead, the patient's desire for esteem could often be exploited to induce good behaviour. Where this did not produce the desired result, 'the general comfort of the patients ought to be considered; and those who are violent require to be separated from the more tranquil, and to be prevented, by some means, from offensive conduct towards their fellow-sufferers'.[177] This could be achieved when 'the patients are arranged into classes, as much as may be, according to the degree in which they approach to rational or orderly conduct'.[178] Such a system had the crucial additional advantage of providing patients with a powerful incentive to exercise self-restraint: the insane 'quickly perceive, or if not, they are informed on the first occasion, that their treatment depends, in great measure on their conduct'.[179]

One of the questions which has puzzled historians of psychiatry (and which also puzzled some of the reformers themselves)[180] was why it took so long to realize that brutal methods of managing the mad were counter-productive, and to discover that lunatics responded better to 'humane' treatment. What was involved here was, I suggest, something more than just the development of a more refined conscience about the conditions the insane were forced to endure. So long as there existed no alternative methods of managing those who would otherwise fail to conform to the rules required for the smooth functioning of an institution, the cruelties of the madhouse keeper were, in fact, functionally necessary. In the absence of better techniques, chains and fear – though crude – were among the only mechanisms which could guarantee at least a minimum of order.

Concentration on the humanitarian aspect of moral treatment has blinded us to the fact that, from a different perspective, it was precisely such a superior way of *managing* patients. For two of what I have just shown were crucial elements in moral treatment, the ward system and the creation of an intimate tie between the patient's position in this classificatory system and his behaviour, are still the fundamental weapons which mental hospitals use to control the uncontrollable. As Goffman puts it,

> since mental patients are persons who on the outside decline to respond to efforts at social control, there is a question of how social control is

176. S. Tuke, *Description of the Retreat*, p. 141.
177. *Ibid.*
178. *Ibid.*
179. *Ibid.*, pp. 141, 157.
180. Compare Ashley's opening words in his speech on the 1845 Lunatics Bill: 'Sir, it is remarkable and very humiliating, the long and tedious process by which we have arrived at the sound practice of the treatment of the insane, which now appears to be the suggestion of common sense and ordinary humanity. . . .' *Hansard's Parliamentary Debates*, Vol. 81, 3rd series, 1845, column 194.

achieved on the inside. I believe that it is achieved largely through the 'ward system', the means of control that has slowly evolved in modern mental hospitals. The key, I feel, is a system of wards graded for the degree of allowable misbehavior and the degree of discomfort and deprivation prevalent in them. Whatever the level of the new patient's misbehavior, then, a ward can be found for him in which this conduct is routinely dealt with and to a degree allowed. In effect, by accepting the life conditions on these wards, the patient is allowed to continue his misbehavior, except that now he does not particularly bother anyone by it, since it is routinely handled, if not accepted, on the ward. When he requests some improvement in his lot he is then, in effect, made to say 'uncle', made to state verbally that he is ready to mend his ways. When he gives in verbally, he is likely to be allowed an improvement in life conditions. Should he then again misbehave in the old way, and persist in this, he is lectured and returned to his previous conditions. If instead of backsliding he states his willingness to behave even better, and retains this line for a suitable length of time, he is advanced further within the quick discharge cycle through which most first admissions are moved up and out within a year. A point is then often reached where the patient is entrusted to a kinsman, either for walks in the hospital grounds, or for town expeditions, the kinsman now being transformed into someone who has the incarcerating establishment and the law to reinforce the threat: 'Be good or else I'll send you back.' What we find here (and do not on the outside) is a very model of what psychologists might call a learning situation – all hinged on the process of an admitted giving-in.[181]

Tuke's invention of these techniques, then, made it possible for the first time to abandon the brutal and overtly harsh methods of management which had previously been inescapably connected with the concentration of large numbers of madmen together in an institutional environment. There was a quite conscious recognition among many of his followers that their central accomplishment was the creation of a new regime of 'curative discipline', and a parallel insistence that '[t]here is a fallacy even in conceiving that Moral Treatment consists in being kind and humane to the insane'.[182] Disciplining the disorderly, as they acknowledged, depended

181. Erving Goffman, *Asylums*, Garden City, New York: Doubleday, 1961, pp. 361–2. (London: Penguin, 1970.)

182. Crichton Royal Asylum, *11th Annual Report*, 1850, p. 39; W.A.F. Browne, 'The Moral Treatment of the Insane', pp. 311–2, and *passim*. To be sure, he insisted that 'kindness' was one element in the equation. But this must not be just 'a barren sympathy. . . . The purely benevolent physician can never be a good practitioner . . . [for one must not] indulge vicious propensities, [or] . . . give way to unreasonable demands . . . [or] be active in soothing momentary pangs at the sacrifice of permanent peace'. (*What Asylums Were, Are, and Ought to Be*, p. 179.) Compare, too, his insistence on 'the necessity for rigid, stringent, even stern discipline among the insane . . .' (Crichton Royal Asylum, *3rd Annual Report*, 1842, p. 21), and his expressed preference for dormitory sleeping

crucially upon the asylum's 'moral governor' taking advantage of the tightly controlled environment the institution provided to oversee and regulate 'the most minute workings of the great moral machine'.[183] Careful and 'discriminate use of ordinary circumstances and trifles in depressing, elevating, tranquillising, rousing, persuading, or governing the insane . . .'[184] could be employed to ensure that 'the impress of authority is never withdrawn, but is stamped on every transaction'.[185]

It was this latent strength of moral treatment as a mechanism for enforcing conformity which came to be emphasized in the vast pauper asylums. There, to the extent that moral treatment was an advance, what it did was to place a far more effective and thoroughgoing means of control in the hands of the custodians, while simultaneously, by removing the necessity of the asylum's crudest features, it made the reality of that imprisonment and control far more difficult to perceive. So that by cruel irony, it was the same central feature of moral treatment which gave it its appeal as a humanitarian reform and which allowed its transformation into a repressive instrument for controlling large numbers of people.

Increasing size inevitably made the doctors more remote, and broke the ties which were supposed to unite superintendent and patient. The daily, sometimes hourly, contact between patient and physician, which the proponents of the institution were arguing was such a valuable feature of asylum life, was already disappearing. Ellis might sometimes refer to 900 patients as his 'family', but the term was clearly a matter of rhetoric rather than reality. He was, it is true, required by the rules at Hanwell to visit each ward and each patient at least once daily, but since the rules also stipulated that he was to prepare an annual report, keep daybooks and casebooks, a record of the name, age, sex, date of admission, occupation, etc, of every patient, act as steward and treasurer of the institution and keep the accounts, as well as oversee every aspect of the operations,[186] his time for this task was naturally limited. The situation had not changed much when the Metropolitan Commissioners visited in 1844:

> The two resident Medical Officers have between them nearly 1,000 patients to attend, and are required by the rules to see every patient twice a day. Each of these officers has an average of 30 patients on the sick list, and above 50 on the extra diet list. Besides these duties, they are required to mix medicines and keep the registers and diaries. Some

accommodation for his charges, since this arrangement 'continues the discipline and inspection exercised during active pursuits into the night, and during silence and sleep. Control may thus penetrate into the very dreams of the insane'. (Crichton Royal Asylum, *10th Annual Report*, 1849, p. 38.)

183. Crichton Royal Asylum, *7th Annual Report*, 1846, p. 35.
184. Crichton Royal Asylum, *11th Annual Report*, 1850, p. 31.
185. Crichton Royal Asylum, *7th Annual Report*, 1846, p. 35.
186. Minutes of the Visiting Justices of the Hanwell Lunatic Asylum, Vol. 2, 1829–31, pp. 364–6, in manuscript at the Greater London Record Office, MA/A/J2.

attention is also required to be paid to chronic cases in which the general health and state of mind are often varying.[187]

Given the increasing remoteness of the superintendents from the patients, 'much authority . . . in large asylums is now necessarily placed in the hands of the attendants'.[188] We have seen that Tuke had emphasized how crucial their role was. Moral treatment depended for its success on the extraordinary devotion and patience of those who were called on to administer it. Especially as asylums began to fill up with incurable cases, 'success' could frequently not be measured in conventional terms. Since the incurables could provide little in exchange for the humane care they received except gratitude (and often not even that), maintaining the requisite morale and dedication among the staff was likely to be difficult, at best. Given the low rewards characteristically offered to those who do society's dirty work, aggravated by the unwillingness of any save the least successful elements of such a class-conscious society as Victorian England to tolerate the defiling close daily contact with the derelict, it proved impossible. Asylums were compelled to recruit their attendants from 'the unemployed of other professions . . . if they possess physical strength and a tolerable reputation for sobriety, it is enough; and the latter quality is frequently dispensed with. They enter upon their duties completely ignorant of what insanity is'.[189] Close supervision and carefully designed bureaucratic rules could coerce a measure of conformity from such an ill-suited lot and ensure the mechanical and perfunctory performance of custodial tasks; but they could not provide the dedicated self-sacrifice, the unflagging interest in the patient's welfare, which moral treatment required. As in other respects, moral treatment could not survive the perils of routinization.

While the reformers continued to use the increased likelihood of cure as one of the primary arguments in favour of asylum treatment, there were already signs that the asylums were filling with chronic cases for whom nothing could be done. Like most of its counterparts, Hanwell was originally planned to be for 'the reception of those only whose malady being of recent date is found to be most susceptible of cure'. In order 'not to fill the asylum with incurables, . . . all pauper Lunatics of these Parishes should be examined, and those Lunatics having the least inveterate symptoms be selected for the County Asylum'.[190] Despite every precaution, however, within three years of its opening, Ellis drew the attention of the magistrates to the 'melancholy fact of the house being filled with

187. Metropolitan Commissioners in Lunacy, *1844 Report*, p. 24.
188. *Ibid.*
189. W.A.F. Browne, *What Asylums Were, Are, and Ought to Be*, quoted in R.A. Hunter and I. MacAlpine, *Three Hundred Years of Psychiatry*, London: Oxford University Press, 1963, p. 868.
190. Middlesex Lunatic Asylum, Minutes of the Visiting Justices, Vol. 2, 1829–31, pp. 323, 412, in manuscript at the Greater London Record Office, MA/A/J2.

old and incurable cases'; something he attributed 'almost entirely to the neglect of proper remedies in the early stages of the disease'.[191] At Lancaster, too, there were complaints of 'the Asylum being crowded with chronic and almost hopeless cases'.[192] And Prichard, the superintendent of the Northampton Asylum, strove to make a virtue of necessity, arguing that

> to express a regret that these pitiable wrecks of intellectual being should find here an Asylum, that during their brief period of mere vegetative existence they should enjoy through its medium every comfort which their unfortunate situation demands, would be as foreign to the dictates of right feeling as to the benevolent views of those by whom it was founded.[193]

To convince his listeners of the advantages of the asylum, Ashley might engage in elaborate and optimistic calculations of the theoretical savings curative institutions would produce. But to do so, he had to ignore the evidence which demonstrated that existing 'county asylums have . . . become, and the evil is daily increasing, places of security rather than curative establishments'.[194] For it is one of the many ironies of the English lunacy reform movement that just as it reached its goals, and the optimism it had been so sedulously promoting reached its peak, experience was showing the fragility of the assumptions on which its whole program rested. There was much truth in the comment made by David Uwins, a London mad-doctor and writer on insanity:

> No well-regulated mind can for a moment doubt that the recent inquisitions by our statesmen and legislators on the nature of insanity and the economy of lunatic establishments have been prompted by a grandeur of design and a largeness of benevolence; the only room for doubt and distrust is in reference to the complete fulfillment of sanguine expectation. It is in the very spirit and nature of reform to be too condemnatory of what is, and too hopeful of what is to come.[195]

191. Hanwell Lunatic Asylum, *4th Annual Report*, 1835.
192. Lancaster Lunatic Asylum, *Annual Report*, 1845, p. 19.
193. Northampton General Lunatic Asylum, *Annual Report*, 1840, p. 12.
194. *Westminster Review* 43, 1845, p. 171.
195. D. Uwins, *A Treatise on those Disorders of the Brain and Nervous System, Which Are Usually Considered and Called Mental*, p. 235, footnote.